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PATIENT'S BILL OF RIGHTS

The Patient and/or Patient's Family has the right to expect and receive the following from Sumner County Emergency Medical Services personnel:

1. A clean, well-maintained, well-equipped, climate-controlled ambulance.
2. The closest available ambulance and first responder (when needed) dispatched to their call for help with no unusual delays for any reason.
3. All responding personnel to be clean, neat, dressed in the appropriate uniforms, and looking professional.
4. All responding personnel to be polite, compassionate, considerate, empathetic, soft-spoken, respectful and well-mannered.
5. Transportation to the clinically-appropriate medical facility of their choice without questions being raised regarding their ability to go by some other means, or any other method of discouraging transport.
6. A clean stretcher with fresh sheets, blankets and a pillow and the appropriate medical equipment brought immediately to their location; carried or wheeled as smoothly as possible while the patient is secured with safety straps and covered with a sheet or blanket to protect the patient's modesty; and wheeled smoothly into the Emergency Department.
7. To have their vital signs checked and documented in both transport and non-transport situations.
8. To have all valuables and medications in their possession documented on the patient care report at the time they are transported, and clearly document to whom in the Emergency Department those valuables and medications were given.
9. To have their pertinent history, injuries, illness, vital signs, treatment, interventions and medications adequately, thoroughly, and truthfully documented on the patient care report.
10. To have the specifics about patient history, current situation and nature of their injury or illness kept confidential in all respects.

COMMUNICATIONS

- Patients, family members, and other persons on scene or in a facility shall be regarded with respect and professionalism. The use of advanced medical terminology should be reserved for those who also possess an understanding of such language and meanings. However, care should also be applied to not speak in a manner that can be perceived as condescending or insulting to one's intelligence.
- When EMS personnel are taking report from others directly involved in the care of the patient, such as care givers, family, or other healthcare providers, this shall be done in a manner that promotes teamwork, mutual respect, and closed loop communication. Vital signs, and other pertinent information given by nursing staff, physicians, or first responders should be recorded in writing when possible.
- It is the responsibility of the most qualified Paramedic caring for the patient to ensure transmission of all aspects of the patient assessment and care to the responding Emergency Unit or Medical Control.
- When reporting a disposition to Medical Control or the responding unit, provide the following minimum information:
 - Identify as Sumner County Medic or Unit with number.
 - Patient's age and chief complaint.
 - Is the patient stable or unstable, including complete V/S and LOC.
 - Interventions performed.
 - Provide other information as requested.
- If a BLS unit is transporting and requests on-line medical control, it is necessary to identify as a BLS unit with no paramedic on board. This will prevent initial confusion from hospital personnel in this rare circumstance.
- For each and every call, the first directives are scene safety and body substance isolation precautions.
- Any confirmed information regarding infectious disease history, or other concerns for contamination to EMS or hospital personnel should be shared between team members involved in the care of the patient as early as possible. (To include known findings of bed bugs, etc.).
- EMS personnel should advise the receiving facility ASAP when transporting a patient with a known or suspected **haz-mat exposure**, in order to facilitate early arrangements for proper decontamination without risking any danger to others. EMS should **NOT proceed into an ER** with any such patient until direction has been given by ER staff.
- Follow the COVID patient management protocol where EMS personnel are directed to communicate with ER personnel to ensure the best approach for delivery of a suspected or known COVID patient into the facility. (Avoid waiting in common areas with such patient on the cot)
- When transporting patients with a suspected acute coronary syndrome / STEMI, EMS personnel should advise the facility of a **"STEMI Alert"** as early as possible.
- When transporting patients with a suspected stroke / CVA, EMS personnel should advise the facility of a **"Stroke Alert"** as early as possible.
- When transporting patients who are critically injured and/or unstable, EMS personnel should advise the facility of a **"Trauma Alert"** as early as possible.
- When transporting patients who are critically ill and suspected to be in septic shock, EMS personnel should advise the facility of a **"Sepsis Alert"** as early as possible.

USE OF LIGHTS AND SIRENS

No emergency response is so urgent that we cannot respond in a safe manner so as to protect the lives of the public and ourselves. To do otherwise could compound an already urgent situation and result in additional emergency patients.

The **safety** of individuals proceeding to the scene as well as the general public is of high priority.

The driver of any authorized emergency vehicle shall not be relieved from the duty to **drive with due regard for the safety of all persons**, nor shall it protect the driver from the consequences of any reckless disregard for the safety of others.

THE LAW

Tennessee Code, Title 55 - Motor and Other Vehicles, Chapter 8 - Operation of Vehicles Rules of the Road 55-8-108 - Authorized emergency vehicles.

55-8-108. Authorized emergency vehicles.

- A. The driver of an authorized emergency vehicle, when responding to an emergency call, or when in the pursuit of an actual or suspected violator of the law, or when responding to but not upon returning from a fire alarm, may exercise the privileges set forth in this section, but subject to the conditions stated in this section.
- B. (1) A driver of an authorized emergency vehicle operating the vehicle according to subsection (a) may:
 - a. Park or stand, notwithstanding other provisions of this chapter that regulate parking or standing;
 - b. Proceed past a red or stop signal or stop sign, but only after slowing down as may be necessary for safe operation;
 - c. Exceed the speed limits so long as life or property is not thereby endangered; and
 - d. Disregard regulations governing direction of movement or turning in specified directions.

(2) Subdivision (b)(1) shall not relieve the driver of an authorized emergency vehicle from the duty to drive with due regard for the safety of all persons, nor shall subdivision (b)(1) protect the driver from the consequences of the driver's own reckless disregard for the safety of others.
- C. **(1) The exemptions granted under subsection (b) to a driver of an authorized emergency vehicle shall only apply when the vehicle is making use of audible and visual signals meeting the requirements of the applicable laws of this state, except that while parked or standing, an authorized emergency vehicle shall only be required to make use of visual signals meeting the requirements of the applicable laws of this state.**

(2) Nothing in this section shall be construed to prohibit the driver of an authorized emergency vehicle, while parked or standing, from making use of both audible and visual signals meeting the requirements of the applicable laws of this state, in the discretion of the driver.
- D. An authorized emergency vehicle operated as a police vehicle may be equipped with or display a blue light only in combination with a red light visible from in front of the vehicle.

[Acts 1955, Ch. 329, § 7; T.C.A., § 59-808; Acts 1986, Ch. 822, § 1; 1989, Ch. 173, § 1; 2001, Ch. 60, § 1.]

USE OF LIGHTS AND SIRENS – GUIDING PRINCIPLES

The driver should be advised by the attending medic if the patient's condition changes while in transport, and the method of operating as an authorized emergency vehicle can be altered as appropriate.

When operating a vehicle as "an authorized emergency vehicle", **both** the warning lights and siren (minimum 100 watt) must be in use. Operating a vehicle with only one of these warning devices in use does not satisfy the requirements of the law / state rules and regulations, and SCEMS SOG's.

There are certain medical conditions that may require the rapid transport of the patient, but without the use of an audible warning device due to the patient's condition (i.e. acute MI, pre-eclampsia, use of stethoscope, or other times in which siren noise distracts patient assessment, increases patient anxiety, etc.). In circumstances where lights only are used for transport, the driver should be advised that the vehicle **cannot** proceed as "an authorized emergency vehicle".

Despite the existence of an emergency situation, there are times when it may be more appropriate to approach a scene or transport the patient to a medical facility without the use of lights and sirens, or 'non-emergency'. Similarly, there may be environmental conditions (i.e. traffic, weather, etc.) in which operating as an emergency vehicle or "emergency" introduces unreasonable risk and/or disruption and provides minimal opportunity to arrive at the scene early. In any case, remember ambulance charges and payment rates do not correspond directly with the use of warning lights and siren. **The term "emergency transport", shall mean "without delay", and does not necessarily mandate the use of lights and sirens.**

When transporting a patient, either "emergency" or "non-emergency", the driver of the ambulance should be especially aware of the physical danger inherent and the operation of an emergency vehicle, and drive in a manner to minimize turbulence to passengers resulting from quick and/or sudden stops, acceleration, and turning movements.

Returning to "The County" or to a station quarters "emergency" for any reason other than an emergency is not allowed. In context with this, responding "emergency" to a hospital to transport a patient to another hospital is also not allowed, unless a physician order is given to do so based on the direct needs of the patient in question.

CALLS ORIGINATING ON HOSPITAL PROPERTY

EMS will still respond no matter where the call originates from. This must be done to ensure all patients get appropriate emergency care either from EMS providing transport, or to confirm that a hospital based rapid response team has made patient contact and no further assistance is required of EMS.

If a patient calls EMS from within the emergency department of a hospital, then they fall under EMTALA laws and must be appropriately transferred by an attending ER physician. Supervisors would need to collaborate with the emergency department physician to ensure EMTALA laws are being upheld while EMS is still offering an appropriate service. These are laws and law enforcement agencies may also need to be involved in cases where persons requesting EMS refuse to comply with hospital staff yet still demand services of EMS.

There is much information on EMTALA. As the industry evolves with increasing call volumes, there are situations arising that create unique circumstances in which the EMS system is abused at times. It is the goal of Sumner EMS to provide good customer service and quality pre- hospital care. This must be done if good coordination with our hospital-based colleagues in compliance with all laws and regulations.

For more information visit: [EMTALA Frequently Asked Questions](#).

RESPONSE – STAGING ON LAW ENFORCEMENT SCENES

Scenes in which EMS may be directed to stage and await clearance of law enforcement personnel often include:

- Domestic.
- Assaults.
- Known weapons involved.
- Active Shooter or Suspect with a Gun.
- Psychiatric, Dangerous Person(s).

EMS personnel responding to these scenes should adhere to the following guidelines:

1. Maintain situational awareness at all times.
2. A non-emergency response is acceptable unless there are confirmed patients with potential life threats. This decision may be left up to the unit commander or command personnel in charge.
3. Position/Stage in a location that keeps you out of the flow of traffic as best possible and at an appropriate distance from the immediate scene.
 - a. (For example, do not stage within seeing distance of a home where a subject is known to be armed with a rifle...)
4. Personnel should remain with the ambulance and ready to move. In the event that the scene expands and a danger is presented before being made secure by law enforcement, personnel shall make all attempts to escape and evade with all due regard possible for the safety of others. YOUR safety is the most important.
5. Maintain communications with dispatch per the appropriate radio channel. Avoid the use of cell phones unless necessary for operational purposes. This can distract personnel from what is going on around them and can draw attention to your immediate position in the dark.
6. When responding to a scene known, or suspected, to involve the potential for violence, personnel should don their body armor **before arriving on scene** if at all possible.
7. Sumner EMS personnel should don their assigned body armor anytime they are responding to a scene known, or suspected, to involve the potential for violence.

RESPONSE – STAGING ON FIRE SCENES

EMS commonly responds with Fire Departments to scenes where patients are not yet confirmed, but likely to present. Some of these scenes include:

- Structure Fires.
- Investigations to odors, smoke, or possible hazardous materials.
- Fire Alarms at places of multiple family dwellings (apartments, hotels).
- Fire Alarms at residential institutions (nursing facilities, etc.).

EMS personnel staging on the scene of a working fire should follow these guidelines:

1. Maintain situation awareness at all times.
2. A non-emergency response is acceptable unless there are confirmed patients with potential life threats. This decision may be left up to the unit commander or command personnel in charge.
3. Position the ambulance in a location that keeps you out of the flow of traffic if possible. Be careful to not position the ambulance in a location that will get you blocked in by fire hoses, other apparatus in a way that would prevent from emergency egress from the scene.
4. Be aware to not position the ambulance where it interferes with fire scene operations due to the layout of large diameter hose, handlines, outriggers on aerial apparatus, etc. To avoid this, it is best practice to position the ambulance slightly away from the immediate scene. Also, position the ambulance and stage in a location that is uphill and upwind of the immediate danger / fire area to avoid hazardous materials such as fumes, smoke, etc.
5. This following equipment should be taken to the definitive staging area:
 - a. EMS stretcher with mounted portable O2 (min. PSI @ 1000).
 - b. Cardiac Monitor / Defibrillator (AED is BLS unit).
 - c. Long Spine Board, consider C-collar and head immobilizer.
 - d. EMS Bag with necessary equipment ready to deliver ACLS/PALS.
6. Report to the Incident Commander and establish communications as best possible. The best location to position the EMS cot with equipment is sideways (to stay out of the way) in front of the primary/first-in pumper apparatus. This is the fire engine with the hoses coming off of it. Even if we cannot hear the radio traffic for the FD at the time, the engineer of this apparatus will be able to. Also, let the Incident Commander know where to find you in the event that a patient is found or a fire fighter has a medical emergency.
7. **DO NOT freelance on scene and STAY TOGETHER.** You could be needed at any moment and need to be at a high readiness level. Treatment and transport of a patient that presents unexpectedly cannot happen without both personnel ready to respond.
8. It will not be routine practice for EMS personnel to assist in fire suppression efforts. This is discouraged as it distracts EMS crews from their primary objective. Furthermore, operating in any role outside of the specified job functions of EMS provider may create liabilities, limitations to worker's compensation coverage, or increase injury potential.

This plan applies to most working fires, where FD operations are in play and the potential for patient presentation via rescued occupants or injured/ill firefighters is likely. If scene conditions or other factors increase a risk toward EMS personnel safety, then it is acceptable to alter this.

USE OF SAFETY GEAR (Personal Protective Equipment / PPE)

Purpose:

Certain scenes will involve increased risks of danger to personnel. This is to emphasize the need to utilize safety gear designed to minimize or prevent injury to EMS personnel.

Plan:

In taking a proactive response to the real potentials of danger facing EMS personnel today, Sumner EMS has made attempts to provide effective safety gear specific to certain missions. The following personal protective equipment (PPE) items shall be used when indicated:

BSI protection gear - Shall be donned as appropriate for the condition of the patient and required interventions at that time.

ANSI Traffic Safety Vests - Shall be worn at all times by personnel on scene of any roadway incident. The vest should be worn on the outside of other clothing to provide high visibility.

Ballistic Vests - Sumner EMS personnel should don their assigned body armor anytime they are responding to a scene that is known, or suspected, to involve the potential for violence.

Universal Ballistic Plate/Carriers - To be worn outside / over ballistic vests to provide additional protection against larger caliber firearms in such situations this is likely.

Extrication Gear - Shall be worn when personnel are actively involved in the rescue or treatment of patients entrapped in a motor vehicle accident. This gear is not designed for structural firefighting and only provides flash protection against fire.

Fire/Rescue Helmets - To be worn along with extrication gear when personnel are actively involved in patient care during extrication of a patient entrapped in a motor vehicle accident. Fire/Rescue helmets should also be worn when in or around scenes involving hazards of structural collapse, or otherwise anytime there is risk to head injury from falling debris.

Personal Floatation Devices (PFD) - Shall be worn anytime personnel are in the immediate vicinity of water deep enough to cause drowning in the event of accidental fall into water. This will include anytime personnel are aboard a boat or other marine rescue / swift water operations.

Goggles / Eye Protection / Helmet face shields - Should be applied when there is risk of glass breaking, flying debris, or when hydraulic rescue tools are being used (due to the risk of lines busting and spraying into the eyes).

Addendum to BSI/PPE - updated 2020 - Masks - Should be worn to prevent the spread and exposure of COVID and other airborne illnesses. Follow the guidelines below:

- Cloth or surgical mask - When directed, use anytime you cannot be farther than 6 feet from others. If the patient is screening negative, a cloth or surgical mask may be used, but if a 6-foot assessment reveals otherwise, then upgrade to a minimum of an N95 mask.
- N95- This is the minimal PPE to be used in a patient screening positive or unknown.
- P-100- This is recommended PPE to use in known COVID or positive screening patients.

BALLISTIC VESTS / BODY ARMOR

Policy:

In accordance with NFPA 1500 and 3000, Sumner County Emergency Medical Services shall provide each full-time field employee with protective equipment that is designed to provide protection from the hazards to which the employee is likely to be exposed and is suitable for the tasks that the employee is expected to perform. Body armor has been proven to provide an effective means to protect the wearer from blunt force trauma, sharp objects as well as gunshot wounds. Protective body armor shall be utilized only by members who are trained and qualified to use such equipment.

Purpose:

The purpose of this policy is to establish procedures for the care, use, and maintenance of protective body armor. Furthermore, to maximize personnel safety through the use of body armor in combination with prescribed safety procedures and to provide personnel with a procedure for proper use and care.

WHILE THE USE OF BODY ARMOR PROVIDES A SIGNIFICANT LEVEL OF PROTECTION, IT IS NO SUBSTITUTE FOR PROPER SAFETY PROCESSES, OR THE USE OF COMMON SENSE.

Issuing of Body Armor:

- All full-time personnel will be issued SCEMS approved body armor when possible.
- All users of the provided body armor will sign a negligent use contract for use prior to wearing.
- Body armor will be replaced if it is damaged or worn out by SCEMS as described later in this document.
- Body armor that is found to be damaged and that needs to be replaced due to misuse or abuse by an employee, will be replaced at the expense of that employee.

Use of Body Armor:

- Personnel should wear body armor anytime they feel it is necessary.
- Body armor shall be worn when dispatched to the following incidents:
 - Any incident where it is reported that shots have been fired, persons have been shot or stabbed, or a weapon is involved.
 - Any civil disturbance.
 - Any reported suicide where the means involve the use of a weapon or are unknown.
 - Any SWAT standby or police situation involving violence.
 - Any domestic violence, family dispute or address that has been identified as a concern for such.
 - Any incident that may potentially involve an explosive device, including a suspicious package, bomb threat, etc.
 - Any time deemed necessary by the IC or officer in charge of unit responding.

Note: This list is not intended to be all inclusive.

BALLISTIC VESTS / BODY ARMOR (CONTINUED) 2 OF 3

- Personnel dispatched to the above types of incidents shall don protective body armor prior to entering the affected area and shall wear said PPE until all potential threats have been mitigated by law enforcement.
- The wearing of protective body armor shall not relieve personnel from the requirement that they stage or remain in a secure area of the incident scene until informed by law enforcement that the scene is secure.
- When not in use protective body armor shall be properly secured to prevent theft.
- Body armor will be worn in accordance with manufactures guidelines and recommendations.
- Personnel will adjust the body armor at the beginning of the shift to ensure proper fit.
- SCEMS owned body armor will not be used outside of shift duty or outside of SCEMS sponsored events.

NOTE: *Please be advised of wearing body armor in extreme heat does require frequent hydration and any attempts to stay cool at all times.*

Body Armor can be removed under the following circumstance:

- Motor Vehicle Accidents where personnel have to enter a vehicle.
 - NOTE: Intent on entering the vehicle for medical/treatment purposes.
- Water Rescues where the employee is:
 - Standing or moving along the shoreline within five (5) feet of the water's edge.
 - When boarding any boat to travel to, or assess a patient.
 - When the responder DONs a personal floatation device (PFD).
 - Anytime the OIC (Officer in Charge) of the incident advises the career staff/member to remove.

Inspection of Body Armor:

- ALL members of command staff will be responsible for ensuring that the body armor is worn and maintained as required by this policy through routine observation and periodic documented inspection at in-services or spot checks in the field.
- Daily inspection of body armor will be conducted for cleanliness, and signs of damage, abuse and wear. This will be done by personnel at the start and end of their shift for their assigned body armor.
- Personnel will report any damage to their chain of command officer, AND the Deputy Chief of Logistics should be notified ASAP.
- Regular inspection of body armor will be completed by command staff or designees.

BALLISTIC VESTS / BODY ARMOR (CONTINUED) 3 OF 3**Care, Maintenance, and Replacement of Body Armor:**

- Personnel may disassemble the outer shell of the vest, wash and dry the outer shell every two (2) weeks in accordance with manufacturer's instructions or as needed daily and then reassemble. The Kevlar portion of the vest does not get washed with any chemicals, other than wiped down with disinfectant wipes.
- Personnel are responsible for proper storage of their assigned body armor.

Research and Development for Future Vest Replacement:

- It will be the responsibility of Sumner County EMS to replace the body armor as needed but no more than every five (5) years, or before the manufacturer's recommended expiration date (based on funding).
- The Deputy Chief of Logistics will be responsible for:
 - Monitoring technological advances in the body armor industry that may necessitate a change in body armor.
 - Assessing weapons and ammunition currently in use in our area, and the suitability of approved body armor to protect against those threats.
 - Working with other administrators to coordinate training that emphasizes safe and proper use of SCEMS purchased body armor.

Guidelines for the Care, Use and Maintenance of the Protective Body Armor:

- The care, use and maintenance of protective body armor shall be in accordance with the manufacturer's recommendations.
- All protective body armor shall meet or exceed National Institute of Justice NIF 0101.06 requirements.
- Body armor shall be correctly fitted to each member using such PPE following the manufacturer's recommendations and shall not be used beyond the manufacturer's warranty period.
- As with any type of personal protective equipment, personnel shall not utilize body armor or bullet proof vests if they have not been trained in its use and limitations.
- Personnel shall not wear improperly sized, ill-fitting, or damaged protective body armor.
- Personnel without body armor are best assigned to tasks that limit their exposure to situations where its use is needed such as RTF teams. All attempts will be made to equip personnel with appropriate body armor to enhance safety while operating on scenes/situations with an increased propensity for violence.
- All questions on the care, use and maintenance of protective body armor shall be referred to the Deputy Chief of Logistics.

BODY SUBSTANCE ISOLATION / BSI PROTOCOL

Purpose:

To emphasize the need to use appropriate protection against communicable diseases when delivering emergency assessment and treatments to the sick and injured.

Plan:

Sumner County EMS personnel, including designated first responder agencies, will provide responders with appropriate protective equipment for body substance isolation. This equipment shall include the following, along with the applications for donning the BSI apparel:

Non-Latex Examination Gloves - Shall be used when making physical contact with sick or injured persons. This is not only to protect the provider, but also the patient... especially those who may have compromised immune systems.

Goggles / Eye Protection / Face shields - Shall be used when assessing or treating patients where there is a likely risk for splash contact into eyes or otherwise onto the provider. Examples include:

- Arterial bleeding.
- Airway Management (suctioning, intubation, cricothyrotomy).
- OB/childbirth.
- Blood draw / usage of Vacutainer vials or other pressurized containers.
- Consider when assessing blood glucose due to risk of accidental splash exposure.

P-100 Mask - Shall be used when assessing or treating HIGH RISK patients who are known or suspected of having airborne communicable diseases such as COVID-19. This is the preferred PPE to use when transporting a known or suspected COVID or another airborne infectious patient.

N95 Filtration Mask - Shall be used when assessing or treating patients known or suspected to have airborne communicable diseases. Examples of this include:

- Respiratory MRSA (in the lungs), Pneumonia, Influenza, Tuberculosis, or COVID.

Surgical Mask - May be *used to put onto the patients* to control the spread of communicable illness through exhalation (TB, respiratory MRSA, etc.)

Gown / Shoe Covers - Shall be worn when indicated to protect EMS personnel from body fluids or other pathogens when assessing and treating situations such as:

- OB/childbirth.
- Trauma Calls.
- **Transport of patients with known infectious disease process.**

Biohazard Bags (Red Bags) - Shall be utilized when disposing of materials saturated or heavily soiled with potentially infectious waste such as body fluids.

Approved Sharps Containers - Shall be utilized when disposing of needles, scalpels, or otherwise sharp objects that pose a danger of puncture or penetration to the skin.

RESPONDING IN INCLEMENT WEATHER CONDITIONS

If there are winter weather conditions that make response dangerous, then personnel should consider limiting the use of lights and sirens. Emergency response often requires the ability of EMS units and other vehicles to come to unexpected stops or slowing down. In the due regard for the safety of other motorists, consideration of responding without lights and sirens is acceptable if personnel feel it may increase accident potential.

When severe weather, such as tornado warnings, are approaching our area or anticipated routes of travel, EMS response/transport should be limited to legitimate emergency scenes or absolute emergency interfacility transfers. At any time that personnel aboard the unit feel that themselves or the patient may be in **imminent** danger, it will be acceptable to wait in the safest location possible and resume response/transport once the immediate danger has passed.

THE DECISION TO LIMIT RESPONSE OR TRANSPORT OPERATIONS DUE TO WEATHER WILL COME FROM SHIFT COMMAND AND REMAINS IN EFFECT UNTIL NOTIFIED BY SHIFT COMMAND TO RESUME NORMAL OPERATIONS.

At no time should personnel drive an ambulance through moving or still water of unknown depth. Flooding can make travel and response extremely dangerous. It takes only 12 inches of flowing water to move a small vehicle. It takes 18-24 inches of flowing water to move larger vehicles. Do not underestimate the power of water. Avoid driving through water and find an alternate route. Request additional resources as needed, but please **do not** risk your safety or the patients by driving through potentially dangerous water on the roadway.

PATIENT REFUSAL OF TRANSPORT (AMA)

Document and Identify the following:

- Presence of Injury or illness and desire for transport.
- Identify the person who made the EMS call.
- Reason for refusal.
- Document mental status to confirm mental capacity to refuse care.
- Legal age of majority or emancipated minor.
- Document mechanism of injury or circumstances of illness.
- Document pertinent past history.
- Vitals signs and problem directed exam.

The following may not refuse transport:

- Patients with impaired judgment and decreased mental status.
- Minors (less than 18 years of age or older unless they are emancipated by the courts).
- All minors must have refusal from parent or guardian, not older sibling or other relative, unless every effort has been made to contact parent/guardian and was not successful.
- Do not release minor on the scene without parent/guardian consent.

Reasons for Non-Transport:

- Minor illness or injury and acceptable alternative transportation available

No Patient Found on the Scene: No person found to have any complaint of injury/illness of any type of degree.

- An ePCR (run report) is to be completed in detail as to why no patient was found, example: no person found on scene, person located with no complaint of injury/illness and denies needing medical assistance.
- If there is any complaint, notable mechanism of injury, or notable nature of illness whatsoever, a refusal of treatment and/or transport should be obtained and witnessed (signatures) if possible.

In order to refuse transport, a patient must be ALERT, ORIENTED, and:

- The patient cannot be somnolent (sleepy, altered mental status).
- Patient must be oriented to time, place, and familiar persons (family, friends on scene).
- Patient should be able to stand and ambulate without deficits or falling.
- Able to understand / comprehend the risks of refusing transport when choosing NOT to allow further care by EMS personnel due to a medical complaint or injury.

Call types where refusals are discouraged, and on-line medical control consult is recommended:

- Pediatric patients - traumas with considerable mechanism of injury.
- Pregnant patients with blunt abdominal trauma - high risk for abruptio placentae.
- Anaphylaxis/allergic reaction calls - rebound anaphylaxis can occur up to 30 minutes later.
- Falls with potential head injuries, especially when patients are on anticoagulants.
- Brief Resolved Unexplained Event - (BRUE) especially in infant/pediatric patients.
- Any patient with concerns of shock, who presents with unstable vital signs.

PATIENT REFUSAL OF TRANSPORT (CONTINUED)

If a pediatric patient / minor appears to need transport for physician assessment, however the guardian / caregiver is refusing to allow transport, contact the on-duty supervisor and involve law enforcement as needed. Avoid threatening anyone with pursuing these actions and do your due diligence to inform people the best you can of why you feel transport is needed at the time. Promote mutual respect and professionalism at all times.

If needed, reference the pediatric refusal protocol and follow the guidelines under requirements to be met.

SPECIAL NOTE:

If a refusal is obtained on a patient who presented with a complaint associated with Paramedic (ALS) scope of practice, then the paramedic should be the provider who documents the refusal.

- Example: Syncope, Chest Pain, any other situations in which the paramedic would be attending the patient in transport should be documented by the paramedic if the call disposition results in a refusal of treatment and/or transport.

DOCUMENTATION OF MVA'S WITH NO INJURIES / NO COMPLAINTS

Upon assessment of persons involved in a motor vehicle accident, if it is determined that the person assessed has NO complaints of injury or illness, and NO signs or symptoms noted by EMS personnel, then a refusal of transport is not required, and this is "No patient found". It is "no patient found" because the person involved is refusing medical assistance due to not having any reason to need it, therefore they are not a patient (as stated in State Protocols). **This shall only be applicable for patients who are their own legal guardian.**

SCEMS personnel should make attempts to acquire the person's name and birth date if the person is willing to give this information. DO NOT ask for social security numbers, as this is not necessary. The person is not being transported and they, nor their insurance will be billed, therefore we do not need social security number information.

Apply the information to the EPCR for the MVA, and return to service when appropriate.

Do NOT include the names or dates of birth for uninjured persons in the narrative of any transported patients. **The information for the uninjured persons goes in its own run number / run ticket.**

In extremis type situations, it is not mandatory to acquire information if the persons involved DO NOT have any complaints of injury or illness and no obvious signs are present. The following are examples of situations in which EMS personnel may forego acquiring specifics on persons who deny injury that are involved in MVAs:

- **Mass Casualty Incidents** that overwhelm immediate scene resources
- **Severe weather** events that pose risk to personnel on scene
- If persons on scene are hostile or uncooperative, **do not risk conflict**
- If there are patients from the MVA who **require immediate transport** due to their condition, DO NOT delay transport to acquire information on non-injured persons involved. Notify supervisor as appropriate.
- If **traffic is posing extreme hazards to personnel safety**, we should not delay clearing the scene to gather information on non-injured persons who have already denied the need for EMS care.

NATURAL DEATH/CRIME SCENE MANAGEMENT

Assessment:

Devastating Injuries / Signs of Obvious Death:

- Decapitation, exsanguination of body fluids, pupils dilated, lividity, rigor mortis, pupils fixed.
- Identify time of when last seen alive, cause, PMHx, medications, and recent Hx of illness or complaints.

Natural Death:

- Ensure PD/SO is on scene or en route.
- Verify and document signs of obvious death.
- Verify by ECG 3 Leads – Asystole.
- Contact shift supervisor ASAP taking all further direction from the on-duty EMS supervisor.

Crime Scene:

- Ensure PD/SO is on scene or en route.
- Personal safety is the first concern.
- Limit EMS contact into to (1) provider to preserve scene/evidence.
- Verify and document signs of obvious death without degrading scene.
- Verify by ECG 3 Leads – Asystole.
- Contact shift supervisor ASAP taking all further direction from the on-duty EMS supervisor.

NOTE:

- Do not withhold resuscitative efforts if care is indicated and signs of obvious death are not present.
- **Only paramedics will document DOA reports when partnered with an AEMT.**



ECG strips confirming asystole are NOT required when there are obvious signs of death such as:

- **Decapitation.**
- **Incineration.**
- **Body Decomposition.**
- **Lividity and Rigor Mortis.**

If there is any question to whether or not there are any signs of life, acquire an ECG!

PHYSICIAN ON SCENE

PURPOSE:

To establish guidelines for determining patient care responsibility at the scene of a medical emergency when a physician is on the scene. The physician must be a licensed health care professional medically qualified to render emergency care in the State of Tennessee who specializes in the care that is needed. It is recognized that a qualified physician on scene may facilitate patient care.

PROCEDURE:

1. AEMTs or Paramedics shall:
 - a. Inform the physician that the AEMT or Paramedics must contact their Medical Control.
 - b. Inform Medical Control of the presence of the physician on scene.
2. Medical Control may:
 - a. Ask to speak to the physician to determine qualifications.
 - b. Request that AEMTs or Paramedics verify the licensure of the physician.
3. Any Physician may offer assistance, but:
 - a. Must provide proof that he/she is a licensed physician with a background in emergency care.
 - b. Allow the AEMT/Paramedic to remain under Medical Control Direction or,
 - c. Request to talk to the base hospital to offer medical advice and assistance or,
 - d. Take **total responsibility** for care given by the AEMT/Paramedic and **physically accompany** the patient until such time that the patient arrives at a hospital and responsibility is assumed by the receiving physician, and shall,
 - i. Sign for all instructions given to the AEMT/Paramedic
 - ii. Maintain Medical Control contact whenever possible.
4. If the patient's private physician is on the scene:
 - a. Inform the patient's private physician that the AEMT/Paramedic must make contact with Medical Control.
 - b. May request that the patient's physician contact Medical Control.
 - c. At no time will the AEMT/Paramedic take any Verbal orders over the phone unless they are coming from Medical Control. The **exception** to this rule is if the physician is known by the AEMT/Paramedic and has rapport with EMS Medical Directors.

HANDLING OF PATIENT'S VALUABLES / PERSONAL PROPERTY

GENERAL STATEMENT

A medic's first responsibility is to treat the patient. Handling a patient's valuables or personal property is secondary to proper pre-hospital emergency care. However, special attention needs to be paid to how a patient's personal property is handled by EMS personnel (when handling it cannot be avoided) to minimize potential problems for EMS later on. In "load-and-go" situations, do not waste time handling patient's valuables.

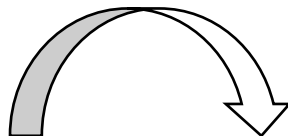
Proper procedure under this protocol is determined by location of the patient (at home, accident scene, etc.), whether family members or friends of the patient are present, whether law enforcement personnel are present and several other factors. **Every situation cannot be described here, but the following is to serve as a guideline.**

Patient's personal property could include but not be limited to: glasses, dentures, wallets, money, watches, jewelry, expensive clothing, medications, and keys.

PATIENT AT HOME OR A RESIDENCE

- Advise and encourage the patient to leave all unnecessary personal items and valuables at home or with a family member or friend.
- A patient's medication in most cases would need to go to the hospital either with the patient or be carried by a family member. If it is necessary for the medic to handle these medications, they should be treated like any other patient valuables.
- Do not remove a watch, jewelry, or wallet from a patient unless it is necessary to treat the patient, (starting an IV, attaching monitor pads, etc...)
- If it is necessary to do so, tell the patient you are removing the item. Then try to give it to the patient if conscious and alert or to a family member if present and document this on the ambulance trip report. If possible, have another medic or law enforcement officer witness what you did with the patient's personal property.
- If the patient insists on taking personal items with them, they must be alert enough to keep possession of the items.
- If you are uncomfortable about the security of the premises you are leaving, notify law enforcement.

CONTINUED ON NEXT PAGE...



HANDLING OF PATIENT'S VALUABLES / PERSONAL PROPERTY (CONTINUED)**PATIENT AT ACCIDENT SCENEN OR NOT AT HOME**

- If the patient is conscious encourage the patient to give personal property and valuables to a responsible person of his choice. If you have to remove any item from the patient (e.g., watch, jewelry, etc.) to treat the patient, return the items to the patient, and if possible, have someone witness this and document it on the trip report.
- If law enforcement presents you with a patient's personal items, request that they (law enforcement) present the items to the patient (if conscious and alert) or to the patient's family, or present them to the hospital staff.
- If personal items or valuables are handled by first responders or bystanders before they were presented to you, document this on the trip report.
- If personal items or valuables are destroyed in order to gain access to the patient, this should be documented and the items kept.
- If patient is disoriented or unconscious give the patient's personal items to a family member or law enforcement officer if possible. Document any incident involving valuables on the trip report and obtain signature from the person receiving valuables. If family or law enforcement are unavailable, transport valuables with patient.

TAKING CHARGE OF PATIENT'S PERSONAL ITEMS

When EMS personnel finds themselves in possession of a patient's personal items and valuables, he/she should carefully document what was done with the items:

- Place the items in a container provided for that purpose – zip lock bags for small items and plastic garbage bags for larger items.
- Make a list of the items placed in each bag and place the list on the bag or in the bag. Medications should be listed separately. Currency should be listed by amount.
- Have your partner or law enforcement officer verify (sign) the list of items included in the bag.
- When you arrive at the hospital, turn the bag(s) over to the appropriate hospital staff (depending on hospital protocol) and have them sign for the items.
- Retain a copy of this signed list to be attached to the EMS copy of the trip report.

SECURING OF WEAPON FOUND ON SCENE / PATIENT**Purpose:**

To comply with Tennessee legislation in regards to concealed carry laws while providing a safe environment for EMS personnel, patients, family members, and members of the public.

Emergency responders and healthcare providers are likely to encounter an increasing number of persons carrying firearms. In most cases, this will be legal carry of concealed firearms, however it is very possible to encounter patients who are illegally in possession of a firearm, yet in emergent need of medical attention. The potential for inadvertent harm to emergency responders as they care for these patients is a concern. Appropriate guidelines and training should be followed to reduce the risk of harm, most significantly, the accidental discharge of a firearm.

Most hospitals are declared “no carry zones” as individuals not exempt (Law Enforcement) are prohibited from bringing firearms into the structure. (T.C.A. 39-17-1359.).

Response Guidelines:

EMS Providers who are licensed to carry a concealed firearm and are doing so at the time of a call, should secure their firearm either at home or in their personal vehicle, prior to entering their station, apparatus, or scene.

Training:

All agencies should obtain appropriate firearm awareness training. This training may be available through their EMS System. Agencies should work with local law enforcement or other appropriate agency to facilitate training their members on basic firearm safety and handling instructions.

Preferred Practice:

EMS providers should make any and all attempts, within reason, to not handle or transport any firearms. If patient condition and safety concerns allow, the patient should secure the firearm at home or in their personal vehicle prior to transport. Law enforcement should be utilized as needed.

Transporting Agencies:

Sumner EMS will need to purchase an appropriate container that can secure a firearm if found. Containers must be in a secure, locked compartment or safe, that ensures the firearm will not have the barrel pointed in the direction of any individuals in the apparatus. We must develop a guideline or policy that outlines how they plan on securing and transporting a firearm if found on a patient. This guideline or policy must be approved by our respective EMS Administrators.

Patient Care During the Initial Assessment of a Patient:

EMS Providers will need to inquire about any concealed firearms. If the patient has a firearm, all legal efforts should be utilized to avoid having to transport a firearm to the Emergency Department. However, if the patient’s condition requires immediate transportation, then transportation should not be delayed unless there is an imminent threat to the providers. If the patient is deemed stable and police are en’route, transportation may be delayed to relinquish the firearm to the police.

SECURING OF WEAPON FOUND ON SCENE / PATIENT (CONTINUED)**Conscious Patients:**

EMS Providers should approach an alert and oriented patient in calm discussion regarding the need to secure a firearm prior to transport. The ideal situation would include the patient leaving the firearm secured at their residence or in their personal vehicle. Simple explanations should be given including current law, as well as this Guideline, if questions arise. If, after this explanation, the patient refuses to secure or relinquish the firearm, EMS Providers may immediately stop their assessment and refuse transportation until Law Enforcement can intervene.

Patients with Altered Levels of Consciousness:

If a firearm is found on an awake patient with an altered level of consciousness, EMS providers should not attempt to have the patient hand over the firearm on their own. EMS providers should not attempt to disarm a patient who they feel would potentially use the firearm against them. Law enforcement should be notified and they should secure the firearm.

Unresponsive Patients:

If a firearm is found on an unresponsive patient requiring immediate care, and law enforcement is not on scene, EMS Providers will need to carefully separate the firearm from the patient prior to transport. If the firearm is in a holster, the entire holster should be removed from the patient. If it is physically impossible to remove the holster with the firearm inside, the firearm will need to be removed from the holster if the provider has received proper training to do so. Once removed, the firearm should be secured in appropriate container that houses firearm that will point away from EMS Providers and the patient.

Transporting Family, Friends, etc.:

All firearm guidelines also apply to anyone requesting to accompany patient(s) to the hospital. EMS personnel will need to inquire about a concealed firearm prior to transport. If the individual refuses to secure the firearm, they should not be allowed to ride in the ambulance.

Safety:

Scene safety remains the top priority for EMS responders. If the EMS Providers feel that there is a valid life threat to them, then retreat to a safe area is indicated. Law Enforcement should be notified and EMS should stage in a safe area until the scene is deemed safe by Law Enforcement.

Notification of the Emergency Department:

If a firearm is being transported to the Emergency Department, EMS Providers should advise, during their radio report that a secured firearm is on board. EMS providers should advise the facility ahead of time that a firearm is on board, and request appropriate security or law enforcement personnel to be on hand to secure the firearm upon arrival. Emergency Departments will follow internal policies on having that firearm secured by appropriate Security personnel or Law Enforcement.

Arrival at Hospital:

Upon arrival, relinquish the secured firearm to appropriate designee as soon as possible. A firearm should not be left unattended at any time.

USE OF AIR MEDICAL TRANSPORT

REQUEST FOR AN AIR AMBULANCE MUST BE IN ACCORDANCE WITH THIS POLICY.

Consider requesting air medical transport:

- A critical medical or trauma patient's condition warrants immediate transport and/or advanced clinical practices or blood products.
- If the patient's condition requires a more rapid mode of transportation to definitive care.
- When time of day, anticipated traffic conditions, and weather make air medical transport more expeditious in transporting a patient to definitive care.
- In critical medical patients (STEMI, CVA, etc.) who present in more rural scene locations.

Air medical Intercept (rendezvous with helicopter at PELA / LZ site) should be used when the air ambulance is not immediately available yet on scene.

Direct loading (aircraft on scene already) into the aircraft should be done on scenes where extrication, rescue, or immediate availability of the helicopter is present.

Crews are encouraged to place an aircraft on standby early after dispatch if call type indicates a possible critical patient.

Crews are also encouraged to be moving toward the appropriate destination when rendezvousing with an aircraft in order to eliminate any delays in patient care.

Additional Trauma Center Criteria:

- Multi-system blunt or penetrating trauma with unstable vital signs.
- Greater than 25% TBSA burns.
- Paralysis or spinal injury.
- Amputation proximal to wrist or ankle.
- Flail or crushed chest.

Situational Criteria:

- High energy mechanisms.
- Prolonged entrapment.
- Multiple casualty incident.
- In cases of any traumatic cardiac arrest, air medical personnel may be requested to the scene to assist in patient care and provide advanced clinical options to the patient (chest tubes, etc).
- In cases of a pediatric cardiac arrest (trauma or medical), air medical personnel may be requested to the scene to assist in patient care. Flight crews may accompany EMS to the ER as needed.
- Careful consideration should be applied before canceling an air ambulance once it has been requested. Supervisors should be consulted prior to doing so when possible.
- Communicate key information about the patient to the incoming air ambulance as soon as possible:
 - Estimated patient weight in kg.
 - Condition of patient (mental status and vital signs)

NOTE: Medical responsibility will be assumed by the flight crew personnel upon arrival at the scene.

USE OF AIR MEDICAL TRANSPORT (CONTINUED)

Previous factors that used to impact transport by helicopter are not as limiting as previously noted. Increased patient size, height, and immobilization devices still allow for air medical transport.

- An appropriate rapid trauma assessment should be done on trauma patients to justify the use of air medical transport. This should be compared to the geographical location, expected traffic conditions, and time frame given for air medical rendezvous. This also applies for high acuity or otherwise unstable medical patients in rural areas. **If you have an index of suspicion for criticality of the patient and air medical transport will be of potential benefit, then do not delay requesting the resource.** *Remember, based on dispatch information from the scene, air medical transport may be put on stand-by if there is a likelihood of criticality or trauma center needs.*
- When air medical services have been arranged by the hospital personnel, EMS will continue to provide transport between the airport and hospital.
- Personnel should be advised that "TIKI MAST" is no longer a routinely available air medical / military support for disaster air ambulance needs. Such resources may be available through Command / State EMS, but are not quickly obtainable in most cases.

SUMNER COUNTY EMS TRANSPORT SERVICE AREA

Our service area is all hospital emergency departments in Sumner County and adjacent counties. That may be altered with the order of an Emergency Physician needing to transfer a patient for a specialty intervention. In such case, any direct order from that physician to transport a patient needs to be carried out without delay. Also, contact your on-duty supervisor.

BASIC LIFE SUPPORT TRANSPORT GUIDELINE

This reference is directed to **interfacility or discharge transports** of patients where there may be concern over the physiological stability of the patient. This does not mandate that personnel cannot or should not use sound clinical judgment to determine stability of the patient. Furthermore, on-hand or on-line medical control through a physician may override this.

Vital Signs Parameters:

Many of the patients transported by Sumner EMS are often unstable, or their condition is subject to change at any given time. These patients may need Advanced Life Support personnel to transport them. For non-emergency/BLS transports, the following assessment parameters should be used as a guide to determine with immediate intervention or consult by the physician attending to the patient is suggested.

Consult with the physician attending to the patient prior to transporting if you have:

- Blood Pressure: >180 systolic.
- Pulse: <60 and symptomatic, > 120 in adults, >150 in children, or >180 in infants.
- **Any question of airway stability or respiratory compromise.**
- Blood Glucose: <50, or >400 mg/dl **and symptomatic.**

NOTE:

If you are transporting a patient being discharged from the "floor" of a hospital, attempt to consult the hospitalist or physician who has been treating the patient last, rather than the ER physician.

- **A Basic Life Support unit should not await ALS intercept when in close proximity to a hospital / emergency room.** It is acceptable to initiate ALS intercept, otherwise a BLS crew should NOT delay transport to await an ALS unit to arrive.
- As patient's condition presents, BLS personnel may initiate all appropriate therapies per protocol/standing orders up to their scope of practice only.
- A report shall be called to the receiving facility as soon as possible.
- If On-Line Medical Control is required, the attending AEMT shall notify the physician that the unit is BLS before consulting for further orders. This lets the physician be aware of the capabilities during transport ahead of further considerations into patient treatments.

PATIENT'S RIGHT TO CHOOSE DESTINATION

Regardless of insurance, established physician or not, and so forth, the patient, guardian, or POA has the right to choose what hospital we transport to. We should function as a patient care advocate and let a patient know if a facility they chose is on diversion, however if they continue to adamantly request that destination, then that is where they should go.

There have been some transports where patients requested transport to a Nashville area facility and crew members have advised patient's or families that due to diversion, EMS could not go there. This is not true. "Diversion" should not be used as justification to transport locally when a patient is requesting transport to a Nashville hospital.

WHEN EMS MAY OVERRIDE THE PATIENT'S / FAMILY'S CHOICE OF DESTINATION:

PATIENT CONDITION:

When a patient is unstable the EMS crew may feel, it is justified to go to the closest appropriate facility. Your documentation would need to reflect your justification for making that choice based on the condition of the patient.

INCLEMENT WEATHER / SAFETY ISSUES:

When transport dangers are increased due to weather conditions, EMS personnel should transport to the closest appropriate facility. We wouldn't want to transport to one hospital, only to do a continuity of care transfer later, possibly after road conditions had worsened. It would be safest for everyone involved to transport the patient one time to the facility that can ultimately meet their needs. You can always contact on-line medical control if you are unsure.

CHOICE OF DESTINATION AGAINST MEDICAL ADVICE:

We do not want patients to refuse transport because of transport destination issues. If a patient is well oriented, and has stable vital signs that indicate physiologic stability, they may still choose to be transported to a farther away hospital even when it is against medical advice. Document these situations appropriately, contact a supervisor as needed, but know that we must function not only as good clinicians, but also as good customer service representatives. We must avoid situations where a **symptomatic** patient refuses our care, especially over transport destination liberties.

DEFINITION OF "CLOSEST APPROPRIATE FACILITY":

Closest appropriate facility references the hospital closest to your location that can provide what interventions your patient will need. For example, if the patient is unstable and requiring immediate physician care, then the closest appropriate facility would be any hospital emergency department. If the patient was stable, however suffering a stroke or AMI, then they need to be transported to a facility that can provide rapid CT or cath lab (PCI) intervention. This is the same for trauma, OB care, etc. This is an overall judgment call on the paramedic treating the patient based on the patient's condition.

WHEN EMS MUST HONOR FACILITY DIVERSION STATUS:

If a hospital is on "CT diversion", then that means they cannot effectively assess and/or treat a CVA patient there without CT scanner capability. Those patients would for sure need to be transported elsewhere. If you are unsure, it may be a good idea to call ahead and consult with on-line medical control to make sure the physician(s) at the hospital you are transporting to can manage the patient at that facility.

Any time a physician gives the order to divert to another facility, EMS must honor that order. Document the name of the physician in the report.

If a facility is on diversion due to "decon", it is possible that they have experienced a biological threat or hazardous materials incident. In such case, they are on definitive diversion and you absolutely should transport elsewhere.

****Veterans Administration (VA) Hospitals may divert patients if they cannot be found in their records. When calling report to these facilities they often ask for the patients initials and last four digits of the social security number to verify that they will accept that patient. Providing this information is NOT prohibited per HIPPA laws. This should be done in advance whenever possible.****

WHEN EMS CAN CONTINUE TO A FACILITY ON "DIVERSION":

EMS should relay to the patient when a facility is on "E.D." diversion due to overcrowding. The patient may still be transported there if they continue to make that choice when stable. If a facility is on "Med- Surg" diversion, it means that the surgical capabilities or admission rooms for post-op care are full and they may require transfer to another facility if surgery is needed. Again, the patient may request transport to the initial choice hospital against medical advice. "Med-Surg" diversion will not apply to all patients, use good judgment and consult on-line medical control as needed.

If the patient is stable and understands the conditions of diversion, yet they still request that facility, EMS should provide transport to that facility. If any problems are encountered due to bringing a patient to a facility in this situation, those should be documented separate from the patient care report and notify your supervisor as soon as possible.

Diversion does not routinely mean that patients cannot be transported to certain facilities. Even complete diversion does not mandate that EMS cannot transport patients to a choice destination, or if that destination is the closest appropriate facility.

MECHANICAL ISSUES DURING TRANSPORT

Safety of personnel and the patient is first and foremost. If a unit has mechanical issues and goes stationary on the side of the roadway, personnel should carefully maneuver the unit to the right side of the roadway. Warning lights should be activated if functioning. If personnel need to exit the unit, traffic safety vests should be worn.

Local law enforcement or TDOT Help Units can be summoned through our dispatch to respond to the location to help divert traffic if needed.

Response should be based on the condition of the patient and traffic dangers that are imposed if the initial unit was forced to go stationary in a dangerous traffic location. In this case, the back-up unit should respond without delay. This may need to be definitively relayed when requesting aid from other services.

Personnel and supervisors should make all attempts to avoid swapping into another unit on the side of the public roadway. The unit should be re-located to a safe position away from moving traffic if possible. This applies when moving the patient into another ambulance or if putting a unit back in service by swapping into a reserve.

TRANSPORT OF PATIENT'S KNOWN TO ABUSE THE EMS SYSTEM

Whereas our goal is to provide excellence in care and good customer services to persons requesting our services, there are individuals in every community who develop a redundant reliance on the EMS system. Issues known to be associated with abuse and/or overuse of the EMS system quickly consume 911 EMS resources. Developed EMS systems must preserve and manage resources such as 911 ALS units for everyone in the community. In order to best manage resources, and stay ready to respond to life threatening medical emergencies; Sumner EMS shall not make it routine practice to accommodate persons requesting transport to any facility beyond LOCAL Emergency Departments for:

- Complaints related to pharmaceutical dependency.
- Patients who are known to EMS, with repeat 911 response for non-emergency complaints of on-going conditions (unless ordered by a physician).
- On-going, non-acute psychiatric illness / disturbance (patients known to EMS).

It is determined that these patients should have an established physician to address their ongoing healthcare needs, especially if their condition is so acute that the patient must request EMS on a regular basis. The patient should be encouraged to establish care through a primary care physician. EMS personnel should transport to local facilities only in these cases.

NOTE:

If patients who fall within these circumstances indeed show signs of acute medical necessity, such as in trauma, stroke, MI cases, then they shall be treated and transported without delay according to the appropriate protocol.

If no acute medical emergency potential exists, yet the patient still requests transport, a BLS unit may respond to the call and facilitate transport. Sound judgment is required in this decision.

---Information regarding persons suspected of overusing / abusing the EMS system should be relayed through the chain of command up to the Office of Clinical Issues for follow up---

- At any time that a patient becomes unstable and requires immediate physician care, EMS should proceed to the closest designated hospital emergency department.
- Otherwise, attempts should be made to accommodate the transportation requests of our patients while using sound judgment that reflects best clinical standards and good customer service.
- Contact the on-duty EMS Command Personnel as needed if there is any question.

EMS TRANSPORT OF SERVICE ANIMALS WITH PATIENTS

This policy is intended to provide information to EMS personnel about the rights of patients and their service animals as well as several of the laws concerning service animals under the Americans with Disabilities Act (ADA). This policy will assist in understanding the rights of patients who utilize service dogs/animals, how these animals should be transported and that these animals have rights under the law that are not granted to domestic pets.

Definition of Service Animals:

The U.S. Department of Justice defines any guide dog, signal dog, or other animal individually trained to provide assistance to an individual with a disability. If the animal meets this definition, it is considered a service animal under the Americans with Disabilities Act (ADA) regardless of whether it has been licensed or certified by a state or local government.

"Guide dog" means any dog that is trained to aid a person who is blind and is actually used for such purpose, or any dog owned by a recognized guide dog training center located within the state during the period such dog is being trained or bred for such purpose.

"Service dog" means any dog that has been or is being individually trained to do work or perform tasks for the benefit of a person with a disability, provided that the dog is or will be owned by such person or that person's parent, guardian or other legal representative

"Person with a disability" as defined by Federal laws, means "Any person who has a physical or mental impairment that substantially limits one or more major life activities; has a record of such impairment; or is regarded as having such an impairment."

The EMS provider may ask the following types of questions when presented with a service animal:

- "Is this a service dog?" or "Does your animal have legal allowances?"
- "Is the service animal required because of a disability?"

The EMS provider may NOT ask about the extent of the patient's disability except as it relates to patient care.

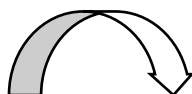
Transporting the Patient and the Service Animal:

When transporting a patient with a service animal, every effort should be made to do so in a safe manner for the patient, the animal and the crew members. If possible, the animal should be secured in some manner in order to prevent injury to either the animal or the crew during transport. Safe transport devices may include:

- Crates, cages, specialty carriers.
- Seatbelts or passenger restraints using a specialized harness or seat belt attachments.
- In certain situations, it may not be possible for the animal to be transported with the patient. In that case every effort should be made to insure safe care and transportation of the animal by alternative means (animal control personnel, family members, etc.).
- EMS should notify the receiving facility of the presence of a service animal accompanying the patient. Additional Information and Resources Regardless of the purpose of the animal, if the animal is a potential threat to health or safety of anyone involved in response, the animal may be excluded from transport.

The following site offer resources for Service Animals: <https://www.ada.gov/topics/service-animals/>

Continue on next page...



EMS and Service Dogs

A quick reference guide to providing safe and efficient care to people traveling with service dogs

The Americans with Disabilities Act (ADA) requires hospitals and first responders to modify their practices as necessary to ensure that service dog users are provided with the same assistance as their peers. **EMS providers must be prepared to safely transport service dogs alongside their handlers.**



The ADA defines a service animal as any dog that is trained to do work or perform tasks for the benefit of an individual with a disability.

- EMS providers may not ask for proof of certification.
- A person may be asked to remove his or her service dog **only** if the dog is out of control or if the dog is not housebroken.
- A service dog is not a pet or companion dog.



There are no regulations to specify where a service dog should be placed during transport. The size of the dog, condition of the patient and space configurations of the apparatus will drive this decision.

A service dog may be placed alongside a center-frame stretcher, remaining clear of the EMS provider. When a patient requires life saving interventions, or if space prevents the dog from transport in the patient compartment, the cab may be a viable option. An alternative vehicle - such as a police car or paramedic fly car - may also be considered.

Created by Ret Capt. Cecilia Warren, MS, MBA in partnership with Guiding Eyes for the Blind
 Photos by Liflander Photography
 Additional materials and presentations available: cw413@comcast.net

Best practices for transporting service dog teams:

- For everyone's safety, the dog should be tethered to a stationary device, i.e., the stretcher or a seatbelt that's locked into place.
- Be sure to secure the stretcher before placing the guide dog in the patient compartment. Remove the guide dog first upon arrival at your destination.
- If possible, place leash on the appropriate collar ring to prevent injury or asphyxiation when securing the dog. (See photo below.)
- Cover sharp surfaces in perforated running boards to prevent lacerations to paws.



Dog's leash has been looped through fastened seatbelt.

When possible, leash should be placed on the "dead" ring before securing the dog.



Sources

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- National Archives and Records Administration (2011). "Electronic Code of Federal Regulations." Title 28. Part 35. *Nondiscrimination on the Basis of Disability in State and Local Government Services.*
- US Department of Health and Human Services Office of Civil Rights. (2011). *Letter of Findings.*
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- US Department of Justice (2011). "Fact Sheet: Highlights of the Final Rule to Amend the Dept of Justice Regulation Implementing Title II of ADA."

Created by Ret Capt. Cecilia Warren, MS, MBA in partnership with Guiding Eyes for the Blind
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HEALTH CARE POWER OF ATTORNEYS

Patients who have a Health Care Power of Attorney retain the right to make healthcare decisions as long as they are mentally capable of doing so. No treatment can be provided or withdrawn against the patient's will unless they become mentally incapable of making decisions.

When transporting patients who are incapable of making their own decisions (unresponsive, altered mental status, special needs patients, or minors) it is best practice to direct any family members, to include the power of attorney, to the ER waiting area if they are present upon arrival or rode to the hospital with EMS. Be sure to let ER staff know that the family/POA is present so they can come be with the patient and stay involved in their care as soon as possible.

EMANCIPATED MINORS

Any non-emancipated minor who is between their sixteenth and eighteenth birthday may consent to health services, but may not refuse. A minor who is married, pregnant (or has been pregnant), or independently living away from home may refuse treatment and/or transport. Refusals of care and/or transport should be well documented in the patient care report (PCR).

MINORS AT SCHOOLS (LOCO PARENTIS)

Legal minors (patients less than 18 years of age) who present at schools may be treated and transported at the request / direction of school administrators, teachers, or school nurses. When responding to a school or to an event sanctioned by the schools, consent to treat and transport is implied by the parents / legal guardians of the student and does not have to be expressed in order to provide treatment or transport. "Loco parentis" applies in these cases and means that the school administrator, teacher, or nurse stands as legal guardian in this situation.

Transport should not be delayed to seek consent from parents or other legal guardians when on scene at a school. A parent may only override this if they are **on the scene** and willing to assume responsibility and guardianship at that time. Otherwise, EMS should transport as agreed upon by school personnel, who have most likely already spoken with the parents/legal guardians beforehand. Make sure to confirm with school personnel and parents/guardians (if possible) where EMS will be transporting to so everyone involved knows where to find the patient once EMS arrives at the designated facility.

DNR, POST, and ADVANCED DIRECTIVES

Policy:

Any patient presenting to any component of the EMS system with a completed Tennessee Do Not Resuscitate (DNR) form shall have the form honored. Treatment will be limited as documented on the DNR form. A living will or other legal document that identifies the patient's desire to withhold CPR or other medical care should be provided in the official forms from the State of Tennessee, otherwise EMS does not have standing orders to honor the document. These official documents for this purpose should be arranged, when possible, in consultation with the patient's family and personal physician.

Purpose:

To honor the terminal wishes of the patient and to prevent the initiation of unwanted resuscitation.

Procedure:

1. When confronted with a cardiac arrest patient, the following conditions must be present in order to honor the DNR request and withhold CPR and ALS therapy:
 - a. The form(s) must be a Tennessee DNR form, or DNR box is checked in section A of the POST form.
 - b. The effective date and expiration date must be completed and current.
 - c. The DNR form must be signed by a physician, physician's assistant, or nurse practitioner.
2. A valid DNR form may be overridden by the request of the patient, the guardian of the patient or an on-scene physician.
3. If the patient or anyone associated with the patient requests that a TN DNR form not be honored, EMS personnel should contact supervisors to obtain assistance and direction.
4. When confronted with a seriously ill patient who is not in cardiac arrest and has a POST form, the **POST form Section B** shall be utilized as follows:
 - a. Full Scope of Treatment box is checked: Use all appropriate measures included in Sumner County EMS protocols to stabilize/resuscitate the patient.
 - b. Limited Scope of Treatment box is checked: The maximum airway intervention is non-rebreather mask and airway suctioning. All appropriate IV medications may be utilized. No electrical therapies are to be provided.
 - c. Comfort Measures box is checked: The maximum airway intervention is non-rebreather mask and airway suctioning. IV pain medications may be administered. Medical Control may be contacted to reference appropriate treatment.
5. If family members are present and ask that resuscitative efforts be withheld in the absence of an advanced directive, determine their relationship to the patient and the patient's history. If the patient has an obvious life-limiting illness (terminal cancer, advanced neurological disease, advanced age, etc.), resuscitative efforts may be withheld. If there is no obvious life-limiting illness, begin resuscitation based on appropriate protocol(s) and contact Medical Control for further guidance.
6. Living wills or other documents indicating the patient's desire to withhold CPR or other medical care may be honored only in consultation with the patient's family and Medical Control approval to not resuscitate.
7. When in doubt – contact supervisors immediately. On-line medical control may be consulted for decisions regarding medical aspects of a pending resuscitation; however, they are not responsible to provide legal consultation toward certain medical-legal aspects of documentation, family disagreement, or other legalities.

Resuscitative Measures to be withheld:

- CPR
- Advanced airway management to include intubation.
- Artificial Ventilations.
- Defibrillation.
- Cardiac Resuscitation Medications.

Approved procedures include:

- Suctioning.
- Basic Cardiac Monitoring.
- Oxygen and Basic Airway (OPA, NPA)
- CPAP.
- Non-cardiac resuscitation medications.
- Comfort Care / Pain Management.

Directions for Health Care Professionals**Completing POST**

Must be completed by a health care professional based on patient preferences, patient best interest, and medical indications.

To be valid. POST must be signed by a physician or, at discharge or transfer from a hospital or long term care facility, by a nurse practitioner (NP), clinical nurse specialist (CNS), or physician assistant (PA). Verbal orders are acceptable with follow-up signature by physician in accordance with facility/community policy.

Person with DNR in effect at time of discharge must have POST completed by health care facility prior to discharge and copy of POST provided to qualified medical emergency personnel.

Photocopies/faxes of signed POST forms are legal and valid.

Using POST

Any incomplete section of POST implies full treatment for that section.

No defibrillator (including AEDs) should be used on a person who has chosen "Do Not Attempt Resuscitation".

Oral fluids and nutrition must always be offered if medically feasible.

When comfort cannot be achieved in the current setting, the person, including someone with "Comfort Measures Only", should be transferred to a setting able to provide comfort (e.g., treatment of a hip fracture).

IV medication to enhance comfort may be appropriate for a person who has chosen "Comfort Measures Only".

Treatment of dehydration is a measure which prolongs life. A person who desires IV fluids should indicate "Limited Interventions" or "Full Treatment".

A person with capacity, or the Health Care Agent or Surrogate of a person without capacity, can request alternative treatment.

Reviewing POST

This POST should be reviewed if:

- (1) The patient is transferred from one care setting or care level to another, or
- (2) There is a substantial change in the patient's health status, or
- (3) The patient's treatment preferences change.

Draw line through sections A through D and write "VOID" in large letters if POST is replaced or becomes invalid.

COPY OF FORM SHALL ACCOMPANY PATIENT WHEN TRANSFERRED OR DISCHARGED.

A COPY OF THIS FORM SHALL ACCOMPANY PATIENT WHEN TRANSFERRED OR DISCHARGED

Tennessee Physician Orders for Scope of Treatment (POST, sometime called "POLST")

This is a Physician Order Sheet based on the medical conditions and wishes of the person identified at right ("patient"). Any section not completed indicates full treatment for that section. When need occurs, first follow these orders, then contact physician.

Patient's Last Name

First Name/Middle Initial

Date of Birth

Section ACheck One
Box Only**CARDIOPULMONARY RESUSCITATION (CPR): Patient has no pulse and is not breathing.**☐ **Resuscitate (CPR)**☐ **Do Not Attempt Resuscitation (DNR / no CPR) (Allow Natural Death)**

When not in cardiopulmonary arrest, follow orders in B, C, and D.

Section BCheck One
Box Only**MEDICAL INTERVENTIONS. Patient has pulse and/or is breathing.**

☐ **Comfort Measures Only.** Relieve pain and suffering through the use of any medication by any route, positioning, wound care and other measures. Use oxygen, suction and manual treatment of airway obstruction as needed for comfort. **Do not transfer to hospital for life-sustaining treatment. Transfer only if comfort needs cannot be met in current location. Treatment Plan: Maximize comfort through symptom management.**

☐ **Limited Additional Interventions.** In addition to care described in Comfort Measures Only above, use medical treatment, antibiotics, IV fluids and cardiac monitoring as indicated. No intubation, advanced airway interventions, or mechanical ventilation. May consider less invasive airway support (e.g. CPAP, BiPAP). **Transfer to hospital if indicated. Generally, avoid the intensive care unit. Treatment Plan: basic medical treatment.**

☐ **Full Treatment.** In addition to care described in Comfort Measures Only and Limited Additional Interventions above, use intubation, advanced airway interventions, and mechanical ventilation as indicated. **Transfer to hospital and/or intensive care unit if indicated. Treatment Plan: Full treatment including in the intensive care unit.**

Other Instructions: _____

Section C

Check One

ARTIFICIALLY ADMINISTERED NUTRITION. Oral fluids & nutrition must be offered if feasible.☐ No artificial nutrition by tube.☐ Defined trial period of artificial nutrition by tube.☐ Long-term artificial nutrition by tube.

Other Instructions: _____

Section D

Must be Completed

Discussed with:☐ Patient/Resident☐ Health care agent☐ Court-appointed guardian☐ Health care surrogate☐ Parent of minor☐ Other: _____ (Specify)**The Basis for These Orders Is: (Must be completed)**☐ Patient's preferences☐ Patient's best interest (patient lacks capacity or preferences unknown)☐ Medical indications☐ (Other) _____

Physician/NP/CNS/PA Name (Print)

Physician/NP/CNS/PA Signature

Date

MD/NP/CNS/PA Phone Number:

NP/CNS/PA (Signature at Discharge)

()

Signature of Patient, Parent of Minor, or Guardian/Health Care Representative

Preferences have been expressed to a physician and /or health care professional. It can be reviewed and updated at any time if your preferences change. If you are unable to make your own health care decisions, the orders should reflect your preferences as best understood by your agent/surrogate.

Name (Print)

Signature

Relationship (write "self" if patient)

Agent/Surrogate

Relationship

Phone Number ()

Health Care Professional Preparing Form

Preparer Title

Phone Number ()

Date Prepared

COMMUNICATING DEATH WITH FAMILY

Policy:

To aid in the notification and grieving process for family and friends after the immediate death of a loved one.

Purpose:

1. Death notification can be very complex and will have lasting impact on family and friends. Understanding the basics of human emotion and the normal reactions to traumatic events can help guide a proper notification.
2. Everyone reacts differently to death notifications; some will appear catatonic with little to no outward emotion while others will become angry and irrational.
3. The five basic stages of grief are:
 - a. Denial and isolation.
 - b. Anger.
 - c. Bargaining.
 - d. Depression.
 - e. Acceptance.
4. **In the pre-hospital environment, it is likely that family will express all five stages of grief rapidly and while EMS is on-scene.**
5. Each individual processes a death notification in their own unique manner. Be patient and courteous and allow the family member to find peace in their own way.

Procedure:

1. Delivery matters:
 - a. Introduce yourself by name and get on eye level with the family member.
 - b. Confirm the identity and relationship of the family member.
 - c. Use a very simple one-line sentence to break the news, "I am very sorry to tell you but (use the victim's name) has died. Do not use "passed", "expired", or "moved on". Also, refrain from referring to the body as a victim, patient, son, etc.
 - d. Immediately inform the family member that they will be helped through the entire process.
 - e. Pause and allow the family member time to process the information and ask questions.
 - f. Demonstrate empathy and understanding.
 - g. Ask if other family members or friends should be notified.
2. **Statements to avoid:**
 - a. I know how you feel.
 - b. You need to be strong.
 - c. Calm down.
 - d. God must have needed him/her more than you.
 - e. Now that you know, I need to know what funeral home you would like.
 - f. It could have been worse.
3. **Helpful Statements:**
 - a. I am sorry.
 - b. This is harder than most people think.
 - c. Is there anyone I can contact for you?
 - d. I wish I could give you an answer that could help, but I just cannot.
 - e. I can't imagine how you must feel, and I am willing to do what I can to help.
4. Lastly, allow the family to speak and grieve. Many times, they just want to be heard.

FIREFIGHTER REHABILITATION

Policy:

Upon arrival on the scene of a working fire, EMS personnel shall report to the Incident Commander and prepare to provide medical assessment, necessary treatments, and be ready for transport if needed.

Purpose:

To provide parameters for normal vital signs and Identify individuals requiring treatment and transport.

Procedure:

1. If safe to do so, position the EMS cot with ALS bag and cardiac monitor sideways in front of the primary pumper apparatus. Notify command of your location and try to establish communications.
2. As firefighter's exit the structure for rehab needs, encourage the removal of all PPE (including bunker pants), rest, cooling, and oral hydration.
3. Assess pulse rate. If greater than 85 percent maximum for age (see note below) perform orthostatic vitals. If pulse rate increases greater than 20 bpm or a systolic B/P drop more than 20 strongly suggest immediate IV hydration and transport.
4. Assess firefighters' exhaled CO₂ levels, especially if they have depleted 2 or more SCBA bottles. This may pose increased risks of re-breathing their own CO₂ during high CO₂ output due to increased aerobic metabolism. This can be done by allowing the firefighter to exhale through a mainstream capnography device, or if supplemental O₂ is needed, sampling may be acquired through a side stream ETCO₂ nasal cannula.
5. If indicated, or if Carbon Monoxide exposure is suspected, assess CO levels, see below.
6. Assessment of vital signs after the responder has rested for 10 minutes after their last exertion.

Abnormal vital signs include:

- a. Blood pressure: systolic greater than 180 or diastolic greater than 110.
- b. Heart rate greater than 120.
- c. Respirations less than 10 or greater than 26 per minute.
- d. **Temperature greater than 101.**
- e. Pulse oximetry less than 90%.
- f. ETCO₂ less than 30-35 mmHg or greater than 45-50 mmHg.
- g. CO greater than 10%.
7. If any abnormal vital signs, strongly suggest rest, rehydration, and active cooling. Re-evaluate in 10 minutes and strongly suggest transport with no improvement in total rehab time of 30 minutes. Report all abnormal vital signs to the on-scene fire incident commander or safety officer.
8. Fire personnel should not be medically cleared to return to full duty with abnormal vital signs.
9. Any person with abnormal vital signs who refuse intervention or return to full duty against medical advice will sign a refusal.
10. Transport will be encouraged automatically for the following:
 - a. Chest pain.
 - b. Shortness of breath unresolved by 10 minutes of high flow O₂.
 - c. Heart rhythm other than normal sinus or sinus tach.
 - d. Syncope, disorientation, or confusion.
 - e. Vital signs not returned to normal limits after 30 minutes of rehab.
 - f. Inability to hold fluids down or vomiting.

NFPA Age-Predicted 85% max heart rate:

AGE	85%
20-25	170
25-30	165
30-35	160
35-40	155
40-45	152
45-50	148
50-55	140
55-60	136
60-65	132

INFANT ABANDONMENT

Policy:

The Tennessee Safe Haven law allows mothers of newborns to surrender unharmed babies to designated facilities within 72 hours of birth without fear of being prosecuted. As long as the baby is unharmed and the child is surrendered within 72 hours of birth, the mother -- or parents -- will not be prosecuted and is assured of complete confidentiality.

The law was enacted to reduce the number of unsafe abandonment of babies.

As of April 2015, 80 children have been safely surrendered in Tennessee since the law took effect in 2001.

Note:

Sumner County EMS stations should not be listed as, or considered as, "Safe Havens" for drop off points of abandoned infants. This is only due to there being no guarantee of personnel being at the station, and likely being on a call... which would leave an infant unsheltered and uncared for.

Purpose:

To provide protection to infants who are placed into the custody of EMS under this law and to provide guidance to personnel when confronted with this issue.

Procedure:

IF an infant is left at an EMS station (we are not acceptable Safe Haven locations due to no guarantee of personnel being at the station):

1. Follow the **General Patient Care Protocols / Pediatric General Guidelines** as needed.
2. Follow the **Neonatal Resuscitation Protocol** as appropriate.
3. Initiate other treatment protocols as appropriate.
4. Keep infant warm.
5. Ensure someone has called (or will call) local Department of Social Services before departing the ER.
6. Transport infant to closest appropriate facility (Peds hospitals have social workers on staff).
7. Document protocols, procedures, and agency notifications in the patient care report (PCR).
8. Attempt to obtain the following:
 - a. Medical information about the baby's parents.
 - b. If possible, name of baby's parents (the person leaving the child does not have to reveal his or her identity).
 - c. Information about the birth.

The Department of Children's Services main office is located in downtown Nashville.

UBS Tower, 10th Floor
 315 Deaderick Street
 Nashville, TN 37246
 (615) 741-9701
 Email: DCS.Custsrv@tn.gov

The Secret Safe Place for Newborns of Tennessee Help Line is 1-866-699-SAFE.

PATIENTS IN POLICE CUSTODY

Policy:

For this policy to be used, the patient only needs to be in the care of police and does not have to be under police custody.

All patients in police custody retain the right to request transport. This should be coordinated with law enforcement.

Purpose:

To assure patients receives appropriate care when being detained by law enforcement.

Procedure:

1. Assess for evidence of traumatic injury or medical illness and follow appropriate protocol.
2. If a Taser has been used, transport to the nearest facility and treat as necessary under trauma care guidelines of "impaled object". EMS is not trained in the removal of taser probes.
3. If a chemical defense spray (FREEZE +P, Mace, Pepper/OC Spray) has been used, irrigate the face and eyes and remove contaminated clothing.
4. Assess for dyspnea, wheezing and a history of asthma or COPD.
5. If patient has a history of a reactive airway disease or shows any signs of dyspnea or wheezing, observe and follow the appropriate respiratory protocol as indicated based on clinical assessment findings.
6. If an asthmatic patient is exposed to pepper spray and released to law enforcement, all parties should be advised to immediately re-contact EMS if wheezing or difficulty breathing occurs.
7. Assess patient for cardiac history, chest pain, or palpitations. If patient shows cardiac related signs or symptoms, follow appropriate cardiac protocol.
8. Continue to observe for agitated delirium syndrome.
9. Agitated delirium is characterized by marked restlessness, irritability, and/or high fever. Patients exhibiting these signs are at high risk for sudden death and should be transported to hospital by ALS personnel.
10. If restraints are necessary, follow **Behavioral Emergencies/Chemical Restraint Protocol**.
11. **Patients restrained by law enforcement devices cannot be transported in the ambulance without a law enforcement officer in the patient compartment who is capable of removing the devices. (i.e. - handcuffs)**
12. If there is any doubt about the cause of the patient's alteration in mental status, transport the patient to the hospital for evaluation.
13. Coordinate disposition with patient, law enforcement and if necessary, Medical Control.
14. Never argue with law enforcement. If law enforcement interferes with the patient's ability to refuse or request care, attempt to obtain a signature verifying refusal or request of care and report the incident to a supervisor.

Special Note:

All persons in the custody of the Sumner County Sheriff's Office (from a scene or from the Sumner County Jail) should be transported to **Sumner Regional Medical Center** whenever possible. If the attending paramedic finds that the patient has an immediate need for alternative transport destination, this should be well documented and based on immediate signs of criticality from a considerable mechanism of injury or nature of illness. If in doubt, consult the attending MD.

TRAUMA ACTIVATION

Policy:

EMS providers shall assess each adult and pediatric trauma patient using the following criteria upon contact.

Once a level 1 or level 2 trauma alert patient is identified in the field through assessment, a crew member must contact the receiving facility as soon as practical and provide a proper report. The words "Trauma Alert" must be included in the report followed by the level of activation (1 or 2).

If a patient refuses transport to a specific facility and is deemed a "Trauma Alert" by the EMS professional, documentation should include the reason for alternate transport destination after explaining the risk of "life and limb" to the patient. (The patient must be well oriented at the time).

Upon arrival in the Trauma room EMS personnel will give a brief report to the trauma team.

Purpose:

To ensure the patient receives the appropriate care following traumatic injuries.

Trauma Criteria Level 1:

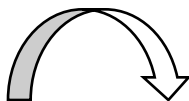
A Level 1 (Full) Alert should be activated on any patient meeting one or more of the following criteria:

- Glasgow coma score (GCS) ≤ 11
- Injury with associated Tachycardia and **poor perfusion**.
- Systolic BP < 90 or for PEDS $\leq 70 + 2x$ (age in years).
- Respiratory rate < 10 or > 29 or when the patient is in respiratory distress, has had a pleural decompression or is intubated.
- Penetrating injury /wound to head, neck, torso, or extremities proximal to the elbow or knee.
- Flail chest.
- All **shotgun** wounds.
- Pelvic fracture to include hip dislocation with significant mechanism of injury.
- Paralysis related to trauma.
- Crushed, de-gloving, mangled, or pulseless extremity.
- Thermal injuries including 2nd or 3rd degree burns $\geq 20\%$ TBSA.
- Electrocution with high voltage: ≥ 220 volts or $>$ than household current.
- Active bleeding requiring a tourniquet or continuous pressure to control.
- Discretion of any trauma team member.

Evidence of Poor Perfusion:

- Skin pallor, cool extremities.
- Weak, distal pulses.
- Cyanosis/mottling, etc.

Continued on next page...



TRAUMA ACTIVATION (CONTINUED)**Trauma Criteria Level 2:**

A Level 2 (Partial) Alert should be activated on any patient meeting one or more of the following criteria after a traumatic event:

- GCS 12 or 13.
- Ejection from automobile.
- Death of a person in the same passenger compartment.
- Falls > 15 feet or > 10 feet (for pediatrics).
- Vehicle rollover with altered GCS.
- High-speed auto crash (collisions with speed 35 mph or greater) **with high index of suspicion.**
- Auto-pedestrian/auto-bicycle injury with significant (5mph) impact.
- Pedestrian thrown or run over.
- Motorcycle crash > 20 mph or Crash with separation of rider from bike.
- Open or multiple fractures, excluding hands or feet.
- Thermal injuries including 2nd or 3rd burns between 10% to 20% BSA.
- Significant neurological deficit.
- Pregnancy > 20 weeks with significant mechanism of injury.
- Discretion of any trauma team member.

High Index of Suspicion / Factors to Consider: (For Activation or upgrade in activation)

- Age > 55 years with significant mechanism of injury.
- Extrication time > 20 minutes.
- Cardiac or respiratory disease.
- Insulin dependent diabetes.
- Cirrhosis.
- Morbid obesity.
- Pregnancy.
- Immunosuppressed patients.
- Patient with bleeding disorder.
- Patients on anticoagulants

AMBULANCE / CREW DECONTAMINATION GUIDELINE

Guideline:

Personnel should be familiar with the Standard Operating Guidelines (SOGs) regarding the reporting of possible exposures. Supervisors shall be contacted as soon as possible to report possible exposures or to request additional time for decontamination procedures beyond routine call types.

Purpose:

To identify a consistent process for allowing adequate time to effectively decontaminate personnel and equipment according to current SOG's and the SCEMS Exposure Control Plan.

Procedure:

In regards to call types where heavy soilage or exposure is a concern, the following process should be followed:

1. **Contact the on-duty supervisor to notify them of the situation.**
2. If there are any injuries or medical complaints of EMS personnel, any decontamination and/or treatment should be acquired as soon as possible upon arrival and delivery of the patient at the hospital.
3. The receiving facility should be notified of the need for decontamination, isolation, and/or potential treatment of EMS personnel in addition to the patient report.
4. EMS personnel should NOT wash out heavy soilage from the rear compartments of the ambulance in the hospital ambulance bays. Decontamination of heavily soiled rear compartments shall be done at EMS stations.
5. Heavy soilage such as blood, emesis, etc. should be wiped or removed as best possible with absorbent materials (towels, etc.) then placed in a red bio-hazard bag if heavily soiled / saturated.
6. After the bulk of heavy soilage has been properly disposed of, an appropriate germicidal disinfectant shall be applied. If additional cleaning is required with a hose to spray out the rear floors, etc., then apply disinfectant prior to flushing the area with a hose.
7. If the Ultraviolet Light (UVL) is needed to properly disinfect the ambulance and equipment, supervisors should be notified and arrangements will be made to accomplish this task while minimizing risks of cross contamination.
8. Appropriate time shall be allotted to afford decontamination of personnel, AND proper cleaning of the ambulance upon notification of the supervisor.
9. **Follow the SCEMS Exposure Control Plan and contact the Exposure Control Officer as needed.**

FORCED BLOOD DRAW – REQUEST OF LAW ENFORCEMENT**Intent:**

It is the intent of the Sumner County EMS to establish a standardized policy that gives direction and guidance to employees while assisting local law enforcement with compliance of mandatory blood draws for blood alcohol as is set forth by T.C.A. 55-10-406.

Scope:

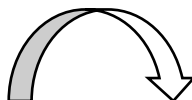
This policy applies to requests generated by law enforcement.

Implementation:

Local Law Enforcement in the course of enforcing T.C.A. 55-10-406 will require blood to be drawn from persons charged with DUI. Sumner County EMS will assist with the blood draw portion of this enforcement. The implementation of this process is as follows:

1. **At no time** shall performance of blood draws under T.C.A. 55-10-406 interfere with patient care or delay transport of any **critical** patient on scene, nor compromise available resources for 911 EMS responses within the County.
2. **At no time** shall performance of blood draws under T.C.A. 55-10-406 be performed if the safety of the SCEMS employees is not certain. The SCEMS employee may refuse to perform the requested blood draw if the employee is not satisfied the procedure can be done without endangering the safety of the employees or the person the blood is to be drawn from.
3. Requests for blood draw assistance will come from law enforcement. An actual call number should be generated by the ECC for this response/special assignment and the CAD record should reflect the time and date of the law enforcement request for EMS to assist.
4. The request for blood draw can be made on scene if the subject is cooperative. If the subject is combative, they may be taken by law enforcement to the Sumner County Sheriff's Office for restraint. With proper restraint and an environment specifically designed for this procedure, it may be safer for EMS personnel and the subject.
5. A written request from Law Enforcement Officers is **not required** if the subject is cooperative and gives consent. This applies on the scene and when on location at the Sheriff's Office facility. If the subject is uncooperative and does not give consent, a warrant must be presented to EMS personnel from law enforcement officers before blood draw procedures are done.
6. The requesting Law Enforcement Agency shall provide SCEMS employee with an in-date, approved Blood Alcohol/Drug Test Kit.

Continued on next page...



FORCED BLOOD DRAW – REQUEST OF LAW ENFORCEMENT (CONTINUED)

7. Sumner County EMS AEMTs or Paramedics will perform the procedure as follows:
 - a. Don appropriate body substance isolation.
 - b. Gather Supply's.
 - 10cc syringe.
 - IV needle.
 - Sharps container if one is not otherwise provided.
 - betadine prep (Do Not use alcohol preps for this venipuncture).
 - gauze.
 - tape/adherent wrap.
 - c. Identify suitable peripheral vein.
 - d. Clean site with betadine (**Do Not use alcohol preps for this venipuncture**).
 - e. Aseptic technique for vein insertion (properly dispose of sharp).
 - f. Draw 10ml blood (equal to 5 ml / blood sample vial).
 - g. Dress insertion site with sterile dressing and tape/adherent wrap.
 - h. Peripheral IV attempts are limited to 3 attempts per EMS provider, and shall be done in the upper extremities only. Vascular access in the legs, feet, E.J., and I.O. are not allowed for this procedure.
8. SCEMS employees performing the blood draw will complete a PCR for the incident. The unit commander on the EMS unit should be sure that any documents provided to EMS are attached to the ePCR before locking the run ticket.
9. The unit commander (Paramedic or senior AEMT) will **ensure that the subject has no signs or symptoms of injury or illness** and be sure this is documented in the ePCR. If there is any complaint of injury or illness, an assessment must be done and a refusal ePCR shall be completed if there is no transport. - per MD1
10. SCEMS employees shall have the requesting law enforcement official sign the ePCR where applicable.
11. Notify dispatch when EMS unit is back in service.

RESCUE TASK FORCE – INCIDENT ACTION PLAN / PROTOCOL

Respond to designated staging area, stay ready, stay with unit.

Move into proximal staging area, once directed.

Prepare gear and establish RTF under direction of senior medic / officer present.

Pair with LEO to create RTF squads 1, 2, and 3 as additional responders arrive.

Establish comms, report to command via assigned TOC, go to Utac 41 before entering as RTF

Link up with LEOs x 2 to make RTF team

First-in senior medic has immediate clinical oversight once at the first casualty

Additional units report to command upon arrival

- Will be assigned to AEP for transport or
- May be assigned as additional RTF teams

Establish CCP**Initiate BATH assessment:**

- Bleeding control.
- Airway.
- Tension Pneumothorax.
- Hypothermia.

Standard **START Triage** to determine who is transport first will apply.

Apply in-direct threat zone EMS treatments:**Bleeding:**

- Tourniquets if applicable.
- Direct Pressure.
- Hemostatic Gauze, pack wounds as needed.

Airway:

- Position.
- NPA.
- Cric (once in safe treatment area).

Tension Pneumothorax:

- Needle Decompression – ARS Needle.
- Occlusive dressing if penetrating chest trauma.
 - Assess / apply dressing to both sides as needed.

Hypothermia

- Keep warm.
- Use blankets to cover patient or as a barrier.

RTF Team 1 focuses on triage and treatment:

- Additional RTF teams will provide evacuation to the AEP for transport.
- RTF team 1 may evacuate themselves if there are only 1-2 casualties.
- RTF evacs to the AEP, patients are loaded into units
- **Transport is initiated immediately. Multiple patients may be loaded on each unit (try to avoid if critical).**

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RESCUE TASK FORCE (RTF) - ACTIVE KILLER / HOSTILE EVENT

RESPONSE PLAN AND PROTOCOL

Background:

This protocol is based from multiple industry standard resources that provide direction on the best practices in mitigating mass casualty incidents involving active shooter/killer threats. EMS functions under the "medical branch" of operations in the ICS layout, specific to NIMS ICS 200. Latest research through the NFPA 3000, original material from the Hartford Consensus, ALERRT training and clinical principles from the NAEMT's TECC guidelines have been developed into a protocol that best suits the needs and capabilities of Sumner County EMS.

Purpose:

The purpose of this protocol is to define the role of EMS in responding to and mitigating MCIs involving a tactical environment. Law enforcement is equipped and trained to "stop the killing", and EMS has joined forces on the immediate scene of any such incident to "stop the dying". It is our duty to best serve our community however possible in such event. This protocol is an evolving document that defines the best practices of "how" EMS does the greatest good for the greatest number in an AS/AK MCI event.

Plan:

Upon dispatch/notification of a MCI scene potentially involving a "active killer", or otherwise confirmed report, EMS personnel shall respond to a SAFE staging area, defined by EMS Command personnel leading the response. EMS Command shall make first contact with law enforcement and establish EMS presence within Unified Command (UC). Responding ambulances and other EMS command will rendezvous at the designated staging area.

Crime Scene Preservation:

- The scene is under the control of law enforcement and will be treated as a crime scene.
- If it doesn't have to be moved, don't!
- Try to remember details on anything or anyone that looks out of place or suspicious.
- Cell phones, etc that are found on scene need to be left so LE can access them...
- Try to not displace fired cartridges lying about.

Interoperability with LEO's:

- LEOs have a high degree of training already, follow their lead and direction in teams.
- They are in a heightened state of awareness, stay with your team!
- "BLUE, BLUE BLUE" is loudly verbalized upon entry of all doors, rounding of corners.
- Do not reach for or grab onto LEOs while operating within the RTF (unless treating them).

Communications:

- Receive dispatch and instruction on EMS Dispatch (primary channel).
- Respond on the assigned **Incident channel** (IDA - Incident Dispatch Assignment).
- Once at staging, advise dispatch you're on scene, going to the assigned **Operations channel**.
- RTF teams operate on operation talk groups (SRO's have radios and may need to relay info).
- Treatment/Transport happens on other operations channels, assigned by EMS Command.
- **Use primary dispatch channel sparingly**, other calls will still be going on during the MCI.

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RESCUE TASK FORCE (RTF) – ACTIVE KILLER HOSTILE EVENT

RESPONSE PLAN AND PROTOCOL (CONTINUED)

Operational Terminology:

- **RTF** – Rescue Task Force.
- **CCP** – Casualty Collection Point.
- **AEP** – Ambulance Exchange Point.
- **LEO** – Law Enforcement Officer.
- **Sit-Rep** – Situation Report.
- **U.C.** – “Command” or Unified Command.
- **AS / AK** - Active Shooter / Active Killer.
- **MCI** - Mass Casualty Incident.
- **START** - Simple Triage and Rapid Treatment.
- **TOC** - Tactical Operating Channel (U-Tac 41, 42, etc).
- **SRO** - School Resource Officer (LEO)

Integration of RTF into MCI operations:

- RTF activation and deployment falls under the IC structure well.
- This is a MCI scene, triage happens at the CCP, treatment here is limited.
- Once in a safe clinical environment (ambulance) or triage area, treatments are started.
- First-in medic unit is the last to leave.
- Pair with other responders to build at least 2 RTF squads, minimal 2 EMS/2 LEO.
- First-in medic/AEMT establishes IDTC triage at CCP, prioritized triage and treatment.
- RTF #2, first to evac.... consider having RTF #3 at the door waiting to evac next patients.
- Resources will dictate specific layouts of teams at different times throughout the incident.

Response:

- Respond to designated staging area, stay ready, stay with unit.
- Move into proximal staging area if directed.
- Prepare gear and establish RTF under direction of senior medic / officer present.
- Pair with LEO to create RTF squads 1, 2, and 3 as additional responders arrive.
- Establish communications.
- Link up with LEOs x 2 to make team.
- First-in senior medic / officer has immediate clinical oversight once at the first casualty.
- RTF#2, 3, and so on... must be prepared.
- Additional medic units report to command, establish AEP, and prepare for patients.
- No stay and play allowed.

EMS SAFETY GUIDELINE

Purpose:

To emphasize the importance of personnel safety through proper situational awareness, following of protocols and laws, and use of safety gear to prevent injury to all persons.

Seatbelt Use:

- All responders, to include students/riders, must wear their seatbelts at all times.
- When transporting a patient requiring emergent treatment, it is understood that this may not be possible. In this situation, extreme caution should be taken by the driver and everyone else to be as careful as possible to avoid sudden braking, sharp turns, etc to prevent injury.
- All providers not actively engaged in patient care should be restrained properly.
- All civilian riders must be properly restrained at all times. If a rider is unable to use the seatbelt properly, they will not be allowed to ride. (Example - unable to properly fit or not enough seats with proper restraints available) - **This may be overridden with supervisor approval only in MCI/disaster situations.**

Patient Safety Restraint / Cot Straps:

- All cot (stretcher) straps should be used at all times. Rare exceptions to this may arise, however it is best practice and should be routine practice for Sumner County EMS personnel to implement the use of all cot straps during all transports.
- If personnel are unable to use the provided shoulder straps, the reason should be documented. Otherwise, the shoulder straps should be used on every transport.

Child Safety Seat Use:

- The built-in child seat in the captain's chair of the ambulance is intended for children greater than one year of age only.
- Parents should NOT hold children/infants in their lap during transport at any time.
- If possible, use a proper safety seat from the parent/guardian's vehicle and secure it to the EMS cot in a fowler's position or in the Captain's Chair.
- Transport of children not being treated as a patient (riders) is best done in an alternate vehicle other than the ambulance.

Securing Equipment:

- The front cab and patient compartment of the ambulance should be free of unsecure objects that could become projectiles in a collision.
- Loose gear or equipment in the back of the EMS unit during transport is not permitted.
- Items such as stethoscopes, radios, or bottles should not hang from the bench seat safety net.
- Portable oxygen cylinders may not be secured between a patient's legs and must be secured in a compartment or in the designed mount on the cot during transport.
- Ventilators and other equipment should be strapped in place with a seatbelt or proper equipment mount when possible.
- At no time should monitors, ventilators, or other equipment be laid on top of the patient's legs.
- Patient's belongings, to include walkers and wheelchairs should be transported in a family member's vehicle and not in the ambulance whenever possible. Supervisors may be needed to help transport these items in a more appropriate vehicle.

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EMS SAFETY GUIDELINE (CONTINUED)**Driving Safety:**

- During nighttime driving, the vehicle passenger should avoid using a computer or other device that yields light reflecting onto the side windows of the vehicle. This can be a critical distraction to the driver and inhibit them from seeing an oncoming vehicle.
- Personnel should not become overly occupied in the use of a GPS, two-way radio, or MDT system at any time while driving the ambulance. These are necessary tasks, but the priority must be safely operating the vehicle.
- When driving emergency and approaching large vehicles such as semi-truck and trailers from behind, note that they cannot use their rear-view mirror with the trailer, and if you cannot see their side mirrors, they cannot see your lights. Sirens are difficult to hear in these vehicles, so being right behind them may not alert the driver to your presence effectively.
- Personnel must always stop for a school bus with flashing red lights activated and a stop sign extended, even when responding emergency traffic. Do not proceed until you are sure it is safe and legal to do so.
- If you come to an intersection or uncontrolled train crossing (no drop arm barriers or flashing lights) and cannot see to your opposite side, maneuver the vehicle sharper toward the direction you cannot see from to improve your angle and visual field. You can also ask your partner to help look if safe to do so. If you cannot see and have no options, ensure the oncoming lane is safe that you can see, enter that lane, then quickly and safely merge into the lane you need to be in. (Red River Road turning left onto Long Hollow Pike intersection is a good example)
- All drivers should yield at pedestrian cross walks; this is State law. When driving emergency, still apply caution in the event a child or other person may still walk out in front of your vehicle.
- The ALS units especially, and the BLS units are top heavy vehicles. The dual rear wheel heavy duty chassis of the ALS units will make it difficult to regain position on a paved roadway surface if the rear tires leave the road. If the vehicle began to turn and slide out of control, it would be possible for the tires to gain sudden traction causing the ambulance to "trip" or overturn. In the event the tires leave the roadway, apply a careful combination of braking and steering to come to a stop or very slow speed. This would be the safest way to regain position on the roadway.
- Be courteous and professional. However, it is not recommended practice to "let people out" into oncoming traffic. We could be held liable for directing another motorist to pull out if this caused them to be in a collision.
- Do not drive over small bridges or culverts if they are not designed to support the weight of a larger vehicle such as an ambulance.
- Avoid driving through water in which you cannot see road surface still. Never drive through water that is deeper than half way up the front tires of the vehicle.
- Avoid letting your emotions lead you to unsafe operation of the vehicle. Remember, other drivers have no idea how bad the situation is that you may be engaged in, and their focus is likely elsewhere.
- When backing the ambulance, always use a spotter when possible. If a spotter cannot be used, perform a "walk around" the ambulance to look for obstacles.

PATIENT LIFTING AND MOVING

Body Mechanics/Lifting Techniques:

Safety Precautions:

- Use legs, not back, to lift.
- Keep weight as close to the body as possible.

Guidelines for lifting:

- Consider weight of patient and the need for help.
- Know your own physical ability and limitations.
- Lift without twisting.
- Have feet positioned properly.
- Communicate clearly and frequently with partner and the other EMS providers.

Principles of Moving Patients:

1. In general, a patient should be moved immediately (emergency (urgent) move) only when:
 - a. There is an immediate danger to the patient if not moved.
 - i. Fire or danger of fire.
 - ii. Explosives or danger of explosion
 - iii. Inability to protect the patient from other hazards at the scene.
 - iv. Inability to gain access to other patients in a vehicle who need life-saving care.
 - b. Life-saving care cannot be given because of the patient's location or position, e.g., a cardiac arrest patient sitting in a chair or lying on a bed.
2. If there is no threat to life, the patient should be moved appropriately, per best practice.

Emergency (Urgent) moves:

In an emergency, every effort should be made to pull the patient in the direction of the long axis of the body to provide as much protection to the spine as possible. If the patient is on the floor or ground, they can be moved by:

- Pulling on the patient's clothing in the neck and shoulder area.
- Putting the patient on a blanket and dragging the blanket.
- Putting the rescuer's hands under the patient's armpits (from the back), grasping the patient's forearms and dragging the patient.
- Never pull the patient's head away from the neck and shoulders.

Non-urgent moves – performed with other responders:

- Direct ground lift (no suspected spine injury).
- Extremity lift (no suspected extremity injuries).
- Transfer of supine patient from bed to stretcher.
 - Direct Carry.
 - Draw Sheet Method.

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PATIENT LIFTING AND MOVING (CONTINUED)**Position Positioning:**

- An unresponsive patient without suspected C-spine trauma should be moved into the recovery position to prevent aspiration if immediate transport is delayed (MCI, etc.).
- A patient with trauma should not be moved until additional EMS resources can evaluate and stabilize the patient with the proper equipment. (non-emergent moves)
- A patient experiencing pain or difficulty breathing should be allowed to assume a position of comfort whenever possible.
- A patient who is nauseated or vomiting should be allowed to remain in a position of comfort; however, the lead attendant should be positioned to manage the airway.

Patient Safety:

- Side rails on EMS cots and beds in facilities must be in a raised position any time personnel are not positioned at the immediate side of the patient. (Fall Risk!)
- Tape, tuck, or otherwise secure straps of spine boards, stretchers, or other equipment before moving a patient. This is to minimize risk of catching or tripping on the straps.
- Do not place oxygen tanks between the patient's legs at any time in transport.
- Equipment (cardiac monitor, etc.) should not be laid on top of the patient's legs.
- All straps, including the shoulder straps of the EMS cot should be used in transport.
- Ensure the patient's extremities (especially hands and fingers) are not resting to where they can be injured while moving through doorways or other narrow passages.

Special Operations / Equipment Notes:

- Stokes Basket - Special Operations Response, follow recommendations and tactics of rescue personnel who are trained in application of webbing or other best practices to secure patients into a Stokes basket in a rescue operation. Ensure the patient is well secured before moving up/down/across rough terrain or in a high angle rescue.
- Be careful when assuming C-spine control of a trauma patient who is floating in water. The patient's body may move with buoyancy. Control C-spine in a manner which will maintain in-line stabilization of the upper body and head/neck at the same time.

Stretcher (cot) Transport:

Per the Tennessee Dept. of Health, Division of EMS, State Protocols, page 117, section 610, SCEMS personnel and responders working under these protocols should transfer and transport patients via the EMS stretcher (cot) when indicated. If a patient refuses the cot AMA or other unique circumstances prevent using the cot / stairchair, document this in the ePCR.

MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION

December 1, 2020 (last updated March 2020)

Clinical Operations Protocol - Update 3.0, effective December 7, 2020

From: The Office of Clinical Issues and Training

Brent Dyer, Deputy Chief - EMT-P

Sumner County EMS

Authorization:

Ray Pinkston, MD

Physician – Medical Director

Greg Miller, Ed.S., EMT-P

Chief of EMS

The following recommendations will apply in regards to treating patients who are suspected of being exposed to or infected by COVID-19. These patients commonly present with ILI (Influenza-like- illness). Here are the signs/symptoms and risk factors for suspected "**high risk**" patients:

- Exposure time frame of 2-14 days prior to S/S developed.
- **Fever** (defined as >100 F), **cough**, and **dyspnea** (presents with acute respiratory illness).

Other reported signs and symptoms:

- Myalgia (body aches) or fatigue.
- Sore throat.
- Headache.
- Cough with sputum or hemoptysis (spitting up blood).
- Diarrhea or nausea prior to fever and development of lower respiratory symptoms.
- A sudden loss of smell or taste has been reported in some cases.

Universal Precautions – Infection Prevention and Control (IPC):

- Wear appropriate PPE and wear it properly.
- Upgrade PPE when in direct contact with a high-risk patient where the provider is in close proximity to the patient's nose or mouth, such as emergency airway procedures.
- MSA Advantage 420 half face respirators are rated P-100 and should be worn anytime providers are subject to be around exposed patients or environments likely to produce exposure.
- **Place a surgical mask on the patient as soon as contact is made.**
- For patients/family that are non-compliant with PPE practices, contact the on-duty supervisor.
- Persons driving the ambulance will remove gloves and any other contaminated PPE prior to entering the cab of the ambulance. A P100 or N95 mask should still be worn as necessary.

General Treatment Considerations and Guidelines:

- Riders will NOT be allowed in the ambulance, with the exception of one parent/caregiver of a pediatric patient. The rider must wear a surgical mask.
- Closely monitor patients with COVID-19 for signs of clinical deterioration, such as rapidly progressive respiratory failure and signs of septic shock.
- Be conservative with IV fluids for high-risk COVID patients when there is no evidence of shock.
- Recognize severe respiratory failure ASAP to better prepare for further interventions.
- Ventilation to perfusion (V-Q) mismatch seen in capnography and pulse oximetry assessment may provide early indication that mechanical ventilation and intubation will be needed.

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MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION (CONTINUED)**Hospital Notification:**

- Notify receiving emergency departments that you are transporting a possible COVID patient as soon as possible, especially when the patient will need immediate aggressive interventions.
- Once you arrive at the hospital, do not enter the Emergency Department like normal. One crew member will need to enter the facility and let the staff know you have arrived.
- Follow all instructions from the ED staff as to where the patient needs to go.
- The only exception to this is if the patient is experiencing a life-threatening condition (CPR in progress, impending respiratory arrest, etc.).

Supplemental Oxygen Therapy:**High Risk of COVID:**

- Give supplemental oxygen therapy immediately to patients who present with respiratory distress, hypoxemia or shock and target SPO₂ > 94%.
- A nasal cannula can be applied under a surgical mask on the patient. If a non-rebreather is needed, place a surgical mask over it for extra precautions.

Adults:

- Unstable adults should receive airway management and oxygen therapy during resuscitation to target SpO₂ ≥ 94%.
- Once patient is stable, the target is > 90% SpO₂ in non-pregnant adults.
- Once patient is stable, the target is > 92-95% SpO₂ in pregnant patients.

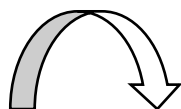
Children:

- Unstable pediatric patients should receive airway management and oxygen therapy during resuscitation to target SpO₂ ≥ 94%; otherwise, the target SpO₂ is ≥ 90%.
- The use of a nasal cannula may be better tolerated in younger children.

Low Risk of COVID:

- Use PPE, follow normal protocols.
- The patient should still have a surgical mask placed on them.
- If a non-rebreather is needed (trauma), the surgical mask can still be placed over the NRB.

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MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION (CONTINUED)

Continuous Positive Airway Pressure (CPAP) Therapy - (HIGH RISK PROCEDURE)

High Risk of COVID:

- CPAP **should be avoided** in patients highly suspected of having or known to be infected with COVID-19. This is primarily due to the need to protect providers caring for the patient by not pressurizing the CPAP system/mask and spreading droplets possibly containing the virus into the air. Some studies have already shown that this can cause droplets to be suspended in nearby environment / air for up to 3 hours.

Low Risk of COVID:

- Carefully assess the increased risk of exposure to providers with how bad the patient really needs the CPAP therapy. Patients, who might need CPAP, will likely be presenting with signs and symptoms that are a high risk of being COVID. Because of this, it is best practice to withhold CPAP therapy, even on low-risk cases, unless directed otherwise by physicians / medical control.
- If possible, the rear doors of the transport vehicle should be opened and the HVAC system should be activated during AGPs. This should be done away from pedestrian traffic. (updated)

Aerosolized Medication Therapy (Nebulizer Use) – (HIGH RISK PROCEDURE)

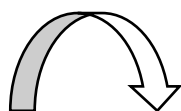
High Risk of COVID:

- The recommendation is to **NOT** use nebulizers (or CPAP) in an ambulance on any patient who is at "**high risk**" for COVID. Nebulizer treatments can place a higher percentage of exhaled droplets/contaminants into the patient compartment, increasing exposure to the provider(s).

Low Risk of COVID:

- If the patient is "**low risk**" for COVID and requires aerosolized medications (**known asthmatic with no recent fever**), carefully assess the increased risk of exposure to providers with how bad the patient really needs the aerosolized medication. Follow these guidelines:
 - Per State EMS, it is recommended to give aerosolized medications outside of the ambulance to prevent exposure to personnel and contamination of the ambulance.
 - You can use the bacterial/particulate filters supplied for ventilator circuits to help minimize exhaled droplet exposure to personnel. (See instructions below)
 - If a nebulizer treatment is absolutely necessary to stabilize the patient, this should be done prior to transport with the rear doors of the ambulance opened and the HVAC system activated. This should be done away from pedestrian traffic. (Updated Dec. 2020)

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MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION (CONTINUED)

Usage of bacterial / viral filters- (To reduce exposure and provide treatments)

- The use of filters while treating the patient in the ambulance is for **low-risk patients** and after intubation of the high-risk patients. Here are various options for use of the supplied bacterial / viral particulate airway filters:
 - Standard Nebulizer - Place either type filter on the distal end of the nebulizer reservoir tube. This will not impede patient exhalation. It will be required that the patient keep their mouth sealed around the mouthpiece to deliver treatment (if absolutely necessary). To do this, let the patient get the therapy as needed without disturbing the process to answer questions, etc. that would make them turn to speak.
 - Bag Valve Mask - Place either type filter between the mask and the self-inflating bag to create a filtration of exhaled air coming back out of the ventilated patient.
 - BVM onto Endotracheal Tube (ETT) - Use the wider disc style filter by Ventlab (REF - FH603003). The larger 22mm connection will go to the BVM and the smaller 15mm connection to the ETT.
 - CPAP system - Use the narrower style filter by Ventlab (REF - BF102). The larger connection (30mm) will go to the CPAP circuit and the smaller connection (19mm) will go to the mask.
 - CPAP system with nebulizer used - You will need to use the same filter noted above for CPAP use, but cannot use the built-in nebulizer port in the CPAP circuit if using a filter. Instead, you will need to place a standard nebulizer proximal to the patient mask, and then place the filter distal of the T-piece connection, with the filter between the nebulizer and CPAP circuit. In order for the patient to receive medication, the nebulizer must be closer to their mask than the filter.

Advanced Airway Management Procedures:

There is a high probability that the virus will be aerosolized during these procedures and constitutes an extremely high risk to the healthcare provider. Use sound judgment and consider less evasive treatments until arriving at the hospital / ER. If the patient's condition **absolutely requires** further intervention at the time, see the following:

High Risk of COVID:

- PPE should include Tyvek suit with hood (minimum of gown), gloves, face shield (eye protection) and P-100 rated respirator (N95 mask if fit tested P100 unavailable).
- Intubation may be performed as needed, and should be done with the video laryngoscope. Only use direct laryngoscopy when you have attempted and failed to intubate with the video device.
- Prep BVM with a filter proximal to the BVM with mainstream ETCO2 adapter in place also.
- At any time, RSI will only be performed with endotracheal intubation and cannot be performed if a supraglottic (King) airway is being used.
- If transport time is short, EMS may await arrival at ER for these procedures to be performed.
- Supraglottic airways (SGAs) may be used as the first-line airway given the current situation.
- ***When intubation is being withheld for arrival at the ER, notify the receiving facility ASAP.***

Low Risk of COVID:

- **PPE should include P-100 / N95 mask, face shield (eye protection), gloves, gown.**
- Intubation may be performed as needed, and should be done with the video laryngoscope. Only use direct laryngoscopy when you have attempted and failed to intubate with the video device.
- Normal protocols should be followed, to include RSI as needed. Filters should still be used.
- At any time, RSI will only be performed with endotracheal intubation and cannot be performed if a supraglottic (King) airway is being used.
- Supraglottic (King) airways may be used on cardiac arrest patients.
- Endotracheal intubation remains the ALS standard of care for "low risk" situations.

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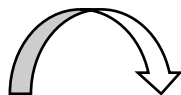
MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION (CONTINUED)**Resuscitation Strategies for Adult and Pediatric Patients with Septic Shock:****ADULTS:**

- In resuscitation for septic shock in adults, give at 250–500 mL crystalloid fluid as rapid bolus in first 15–30 minutes and reassess for signs of fluid overload after each bolus.
- In adults, administer vasopressors when shock persists during or after fluid resuscitation. The initial blood pressure target is MAP $\geq$ 65 mmHg in adults and improvement of markers of perfusion.

PEDIATRICS:

- In resuscitation from septic shock in children, give 10–20 mL/kg crystalloid fluid as a bolus in the first 30–60 minutes and reassess for signs of fluid overload after each bolus.
- In children, consider vasopressors if signs of shock are present:
 - Altered mental state.
 - Tachycardia - Infant HR >160 /min or Child HR >150 /min.
 - Bradycardia - Infant HR >90 /min or Child HR >70 /min.
 - Tachypnea.
 - Mottled or cool skin or petechial or purpuric rash.
 - Age-appropriate blood pressure targets are not achieved.
 - Signs of fluid overload are apparent (liver engorgement).

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MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION (CONTINUED)**Interfacility Transfer Considerations**

The following recommendations pertain to the INTUBATED mechanically ventilated patients.

- Be sure to approach the patient for transfer with a minimum of 1000 PSI in the portable oxygen tank. This is to ensure there is an adequate oxygen supply to transfer out to the EMS unit. Likewise, be sure there is a minimum of 1000 PSI in the portable tank upon arrival at the destination facility in case it takes longer than expected to relinquish care of the vented patient.
- Consider using forceps to “clamp” the ETT to prevent the loss of PEEP and release of exhaled contaminated air during the transition from hospital to EMS vent circuits.
- Have the EMS transport vent set, circuit attached, and ready for a quick transition.
- Many of the patients diagnosed with COVID present with excess secretions and may require frequent suctioning. If a closed suction catheter with airway adapter is already attached, keep it in place for use in transport.
- Whereas research is still limited, many COVID patients who are on ventilators are being managed similar to patients with Acute Respiratory Distress Syndrome (ARDS). This includes:
 - Higher levels of PEEP to maintain adequate oxygenation.
 - Lower tidal volumes, and higher rates to maintain adequate minute volume.
 - Higher level of FiO₂, with a goal of minimum required FiO₂.
 - Increased "I:E ratio" - (inhalation / exhalation ratio).
 - Minimum plateau pressures and driving pressures may be ordered by the physicians.
- Paralytics are recommended in these patients. This is to prevent aspiration, coughing, self-extubation, and patient movement that could cause additional contamination.
- Both the use of paralytic medications and increased levels of PEEP may decrease blood pressures, and special attention must be given to monitor for and prevent any hypotension.
- Implement mechanical ventilation using lower tidal volumes (4–8 mL/kg predicted body weight, PBW) and lower inspiratory pressures (plateau pressure < 30 cmH₂O).

ADULTS:

- The initial tidal volume is 6 mL/kg **predicted body weight (PBW)**; up to 8 mL/kg.
- Permissive hypercapnia could be ordered/implemented.
- Sedation may be required to control respiratory drive and achieve tidal volume targets.

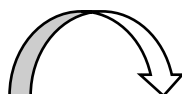
PEDIATRICS:

- Tidal volumes should be adapted to disease severity: 3–6 mL/kg PBW in the case of poor respiratory system compliance, and 5–8 mL/kg PBW with better preserved compliance.
- In children, a lower level of plateau pressure (< 28 cmH₂O) is targeted.
- Permissive hypercapnia could be ordered/implemented. A lower target of pH is permitted (7.15–7.30).

Consult with medical control to consider pronation of the patient as a final option!

(In the critical patient in which the PEEP and FiO₂ should no longer be increased)

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MANAGEMENT OF PATIENTS WITH COVID-19 EXPOSURE OR INFECTION (CONTINUED)

Special Considerations:

- At this point, there is no evidence that pregnant women present with increased risk of severe illness or fetal compromise.
- In the event of a field OB delivery, skin to skin contact with baby/ mother is acceptable.
- Breastfeeding may be facilitated as normally done.
- COVID can be a threat to mother and baby if exposed. Consult with receiving facility to develop the best plan of action to minimize any exposure.

This update may be considered protocol / standing order effective immediately. Personnel will be provided further updates as recommendations and orders come from our Medical Director and other officials as new information develops. **Our focus is to protect the wellbeing of our providers as valued members of the healthcare community.** We are also proud to research and develop protocols and acquire necessary equipment that allows us to maintain a high standard of care to the patients we serve while minimizing the risks to all personnel. Recommendations are welcome and should be sent to the Office of Clinical Issues and Training for consideration.

-BND (DC910), authority of MD1.

References:

<https://www.cdc.gov/coronavirus/2019-ncov/hcp/clinical-guidance-management-patients.html>

[https://www.who.int/publications-detail/clinical-management-of-severe-acute-respiratory-infection-when-novel-coronavirus-\(ncov\)-infection-is-suspected](https://www.who.int/publications-detail/clinical-management-of-severe-acute-respiratory-infection-when-novel-coronavirus-(ncov)-infection-is-suspected)

http://www.ardsnet.org/files/ventilator_protocol_2008-07.pdf

NFPA Emergency Preparedness tools/link:

<https://www.nfpa.org/Codes-and-Standards/Resources/Standards-in-action/NFPA-responds-to-the-coronavirus>

NFPA 1581 - Standard on Infectious Disease Prevention in Fire/Emergency Services

--References below added with December 2020 update--

<https://www.cdc.gov/coronavirus/2019-ncov/hcp/guidance-for-ems.html> (June 2020)

BILLING OF NON-TRANSPORTS AND DOA RESPONSE

Policy/SOG update, approved by Chief Miller, Jan 18th, 2022 - ready for distribution per 901.

Billing for EMS Response Involving Death:

EMS may bill for responses and in some cases transports that include death of a patient. Medicare ambulance benefits are transport benefits. If no transport of a Medicare beneficiary/patient occurs, then no Medicare payment is allowable, except in the case of death pronouncement by a qualified professional (EMTP, or other delegate of the ME) **prior to arrival at a hospital**.

EMS reimbursement in these cases is primarily based on the chronological sequence of events surrounding the death and associated EMS response. Therefore, it is vital the Patient Care Report (PCR) reflects a truthful and accurate chronological depiction of the situation. The following scenarios describe where Medicare and other insurance companies may reimburse EMS response involving death:

- When EMS arrives on scene and confirms death per the Field Determination of Death Protocol on page 82 of the Adult/General Protocols, the specific time that death was pronounced will be documented and the supervisor notified. This is a situation where EMS will bill for services rendered whether the patient is transported or not. Insurance information should be obtained along with demographics when the time is most appropriate with an empathetic and compassionate approach.
- In the event that a separate ambulance is called to transport the body, the unit that responded to the initial call (ALS unit, paramedic on scene) shall be responsible for obtaining the information and completion of the ePCR.
- If a BLS unit later responds to provide transport of the deceased, a BLS ePCR shall also be completed by whoever is assigned to that unit (EMT-B or AEMT).

Billing for Treat and Release Non-Transports:

Sumner County EMS will bill for treat and release (as defined below). It is always the best practice to transport a patient to an appropriate Emergency Department for physician assessment if the nature of illness or mechanism of injury involved required treatments to be initiated or if the patient requests transport. The following treat and release call dispositions will be billed for by Sumner EMS:

- Treatment initiated on scene or during transport that involves the administration of drugs or therapies will be billed for by Sumner County EMS. However rare, cases may arise where a patient has treatment initiated, improves or changes their mind in transport, and adamantly requests to be released. This will be well documented and a refusal AMA will be completed. The patient will receive a bill for services rendered in these cases.
- Examples of treat and release call dispositions that are associated with a refusal of transport against medical advice are:
 - Hypoglycemic/Diabetic Emergencies requiring oral glucose or IV Dextrose administration.
 - Opioid overdose requiring the emergency administration of Narcan (Naloxone).
 - Allergic reactions/Anaphylaxis requiring the administration of emergency medications.
 - Respiratory or cardiac related emergencies requiring medication administration.
 - IV access that was established for fluid or medication administration.
- Special attention shall be applied by EMS personnel to ensure clinical operations protocols are followed and physician orders are obtained for any refusal in AMA (Against Medical Advice) situations where standing orders are not in place.
- The level of provider who attends the patient in transport according to scope of practice requirements shall be responsible for obtaining the information and completion of the ePCR.

Reference: Chapter 10, Section 10.2.6 of the Medicare Part B Benefit Policy Manual.

RESTRAINT PROTOCOL

Protocol:

- Responder safety is a priority while delivering compassionate and empathetic care to those suffering from a behavioral health crisis. Ensure scene safety.
- Involve law enforcement when needed. In most cases, law enforcement will be on the scene already. Stage and await law enforcement clearance when indicated.
- Maintain a high level of situational awareness and avoid letting your **exit** from a room or other means of **egress** becoming blocked in the event the patient's presentation suddenly escalates or becomes dangerous.

Assessment and approach:

- As always, assure the ABCs are intact, stabilize as needed. Apply O2 as indicated.
- Approach the patient slowly and talk in a calm and reassuring tone.
- Protect patient modesty and avoid making the patient feel trapped in a room or enclosed area.
- Verbal de-escalation is the safest method and should be delivered in an honest, straightforward, friendly tone avoiding direct eye contact and with respect to the patient's personal space needs.
- Consider diverse social, cultural, or religious backgrounds of the patient and promote mutual respect.

Restraint Protocol (Physical Restraint):

Any patient who may harm themselves or others may be restrained to prevent injury to responders, bystanders, or the patient. Restraining must be performed in a humane manner and used as a last resort. This should be done with law enforcement personnel present. ***Use the chemical restraint recommendations mentioned in the yellow box below or in the "Psychiatric Emergencies" protocol when safe to do so.***

- Ensure that there are sufficient personnel available to physically restrain the patient safely.
- If possible, consider restraining (if using soft restraints) the patient to a long spine board instead of the cot, so they can be moved easily to the ER bed upon arrival at the receiving facility.
- Restrain the patient in a lateral or supine position. No devices such as backboards may be placed on top of the patient. Never "hog-tie" a patient. In order to gain control, the patient may need to be in a prone position, but must be moved to supine or lateral position as soon as possible.
- The patient must be carefully monitored at all times. This includes direct visualization of the patient, ECG to monitor heart rate, pulse oximetry, and waveform capnography monitoring whenever possible.
- EMS personnel should use soft restraints (cravats, Kerlex, or other approved restraints) when possible.
- If a patient is transported in handcuffs, a law enforcement officer must accompany or escort EMS in transport.
- Extremities that are restrained should have a circulation check at least every 15 minutes along with vital signs.
- Once applied, restraints should not be removed in the field unless medically necessary to provide care.

Chemical Restraint (sedation) – PARAMEDIC ONLY

It may be necessary to provide chemical restraint by administering sedation to a patient who is experiencing excited delirium or exhibiting such aggressive behavior that they will harm themselves or continue to pose a risk to responders. It may require physical restraint in order to deliver chemical restraint safely.

For the acutely agitated and dangerous patient, consider **Versed (Midazolam) 5 mg IM, or 2 mg IVP**

Follow-Up to Restraint Protocols:

- Obtain IV access, if possible, only if safe to do so without risking safety of providers or the patient.
- **If possible, determine the patient's blood glucose level and treat as indicated.** Document in ePCR.
- Causes of combativeness may be due to **underlying medical conditions** such as, hypoxia, hypoglycemia, drug and/or alcohol intoxication, drug overdose or brain trauma. Consider and treat as indicated.
- Documentation should include the reason for the use of restraint protocol, the type of restraint(s) used, the time restraint(s) were used, circulation checks, and other general assessments (vital signs, etc.).